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TO DO NO HARM: STRATEGIES FOR PREVENTING PRESCRIPTION DRUG ABUSE

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HEARING

before the

SUBCOMMITTEE ON CRIMINAL JUSTICE,  
DRUG POLICY AND HUMAN RESOURCES

of the

COMMITTEE ON  
GOVERNMENT REFORM

HOUSE OF REPRESENTATIVES

ONE HUNDRED EIGHTH CONGRESS

SECOND SESSION

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# TO DO NO HARM: STRATEGIES FOR PREVENTING PRESCRIPTION DRUG ABUSE

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MONDAY, FEBRUARY 9, 2004

House of Representatives,  
Subcommittee on Criminal Justice, Drug Policy and  
Human Resources,  
Committee on Government Reform,  
Winter Park, FL.

The subcommittee met, pursuant to notice, at 9:07 a.m., in  
the Winter Park City Hall, 401 Park Avenue South, Winter Park,  
FL, Hon. Mark Souder (chairman of the subcommittee) presiding.

Present: Representatives Souder, Mica, Norwood and Keller.

Staff present: Nick Coleman, professional staff member and  
counsel; and Nicole Garrett, clerk.

Mr. Souder. Good morning, and thank you all for coming.  
This hearing focuses on a very old and very widespread problem,  
the abuse of prescription drugs. Prescription drug abuse itself  
is nothing new, but recently a new generation of morphine-based  
pain killers has caused a wave of addiction in overdoses  
throughout the United States. The drug OxyContin has produced

the greatest amount of publicity, but numerous similar drugs such as, Percocet, Percodan, and Tylox have also been abused.

Prescription drug abuse presents special problems for the government, the medical community, and the pharmaceutical industry. On the one hand, these are powerful and dangerous drugs, with as great a capacity for addiction and abuse as heroin and cocaine. There are many ways for these drugs for to fall into the wrong hands. Supplies of the drugs can be stolen from pharmacies and manufacturers, and then sold on the black market. Doctors may intentionally or unintentionally over-prescribe the drugs to patients leading to addiction and abuse. Or patients themselves may obtain illegal quantities of the drug by shopping for multiple prescriptions and filling them at multiple pharmacies.

On the other hand, these drugs have legitimate medical uses, and may give the only possibility of relief for patients suffering from chronic pain. Many cancer patients for example, rely on OxyContin and similar drugs to combat crippling pain, while other individuals suffering from severe injuries may need similar treatment. Any regulatory plan must balance these completing concerns. Two Federal agencies are primarily responsible for the regulation of prescription drugs. The U.S. Food Administration and the Drug Enforcement Administration.

The FDA has the job of testing new drugs, and specifying how the drug may be marketed, prescribed and used, while DEA is responsible for monitoring the distribution and prescription of these drugs to prevent their illegal use. In addition to investigating illegal trafficking of prescription drugs, DEA also, controls the licenses that every physician must have in order to prescribe controlled substances. FDA and DEA have been criticized both for being too lenient and for being too strict in the regulation of prescription drugs.

Former addicts, relatives of those who have died of overdoses and many media commentators have argued that FDA has failed to safeguard the public from dangerous drugs by sufficiently regulating their marketing and distribution. These critics, some of whom it must be noted have filed lawsuits, have accused manufacturers of over-marketing pain killers and failing to warn doctors of the real risks of addiction and abuse.

By contrast, some doctors, patients, and other advocates for pain treatment have accused DEA of carrying out a virtual war against physicians by aggressively prosecuting those who willfully over-prescribe pain killers. While the specific actions of FDA and DEA and the pharmaceutical companies may be debated, it is clear that the Federal Government needs to explore new approaches to these problems. Congress and the executive branch need to reexamine the approval and marketing process, and determine how best to monitor the distribution and state of pain killers.

Several new proposals are already being debated. For example a number of States are exploring the concept of setting up computerized data bases that would track the sale and prescription of controlled substances to enable law enforcement officials to determine when a doctor is prescribing, a pharmacist is dispensing, or an individual is receiving suspiciously large amounts of a drug. Many States are also attempting to combat the illegal distribution of these drugs over the Internet, an issue that Government Reform Committee Chairman Tom Davis is working to address.

Other proposals focus on what warnings pharmaceutical manufacturers are required to give doctors and patients in providing information on addiction and how to treat it.

This hearing will allow the subcommittee to hear from governmental, medical, and other witnesses to testify about the cost of prescription drug abuse, the benefits afforded by those drugs, and how to best balance between these two.

I first want to thank Congressman Mica for proposing this hearing, and for the assistance that he and his staff have provided in setting it up. Congressman Mica, was chairman of this subcommittee before myself, and both of us have been active on this committee since the Republicans took over Congress. In fact Congressman Mica, used to be, in his first term, a critic of this subcommittee for not focusing on drug abuse and when we took over Congress this committee changed from having I think maybe one hearing on the issue on illegal drug use to becoming the focal committee in Congress. Then, now Speaker Hastert, chaired the committee with Congressman Mica being a very active member, and then Congressman Mica chaired it, and it has been my honor to chair it since then. And he has been vigilant from the time he was a staffer for Senator Hawkins as I was for Senator Coats and we worked on these issues in 1989 and 1990, to coming over as we became the majority in the House and making sure we have both the best health care system in the United States, but also go after the illegal drugs in the United States. I appreciate his coming to me on the House floor saying we need to focus on this and I really would like you to do this in Florida, and for his leadership in the House on this issue.

We also have been joined by two of my colleagues, Congressman Charlie Norwood who also came in with our class in 1994 and we have been good friends for a long time, and Congressman Keller from Florida who is a more recent Member of Congress who we served on the Education Committee together, and have since moved over, and who has been another leader in Congress.

We also welcome three witnesses who joined us to discuss the Federal Government's response to this problem. Mr. William T. Fernandez, Director of the Central Florida High Intensify Drug Trafficking Area or HIDTA, a program administrated by the White House Office of National Drug Control Policy; Dr. Robert J. Meyer, Director of the U.S. Food and Drug Administration's Office of Drug Evaluation at the Center for Drug Evaluation and Research; and Mr. Tom Raffanello, Special Agent in Charge of the Drug Enforcement Administration's Miami Office.

We are also pleased to be joined by two representatives of the Florida State government who have taken a lead role in fighting against prescription drug abuse, Mr. James R. McDonough, director of the Florida Office of Drug Control; State Senator Bert Saunders, who has just called in and has had an emergency and cannot be here.

We also welcome Dr. Stacy Berckes, Board member of the Lake Sumter Medical Society; Mr. Jack E. Henningfield, of Pinney Associates who is testifying on behalf of Purdue Pharma; Ms. Theresa Tolle, president of the Florida Pharmacy Association.

We also, welcome several witnesses who can discuss the importance of these issues to patients and individuals. In particular, we welcome Mr. Frederick Pauzar, who lost a son to an OxyContin overdose, and who has taken a leadership role in addressing the problem of prescription drug abuse. We are

especially pleased to be joined by a specialist in the treatment of prescription drug addiction, Dr. Douglas Davies, medical director of the Stewart-Marchman Center. We also, welcome Professor Paul L. Doering of the University of Florida College of Pharmacy; Ms. Karen O. Kaplan, president and CEO, of Last Acts Partnership, and Dr. Chad D. Kollas, medical director of the palliative medicine at M.D. Anderson Cancer Center of Orlando.

We thank everyone for taking the time to join us this morning, look forward to your testimony, and now I would like to yield to my friend and colleague Mr. John Mica.

Mr. Mica. Thank you, Mr. Chairman. I am pleased that the Subcommittee on Criminal Justice, Drug Policy and Human Resources has agreed to conduct this first oversight hearing on the problem that we face not only in our community and our State but also our Nation, the problem of misuse and abuse of certain prescription drugs, particular today we are going to focus on the problem of OxyContin abuse and misuse. I think this is a very important hearing, and I appreciate your responding to my request.

I want to also thank and welcome Charlie Norwood, from Georgia. A gentleman from Georgia, he is a key player in this. Our committee is investigative and oversight. Dr. Norwood--and he has a medical background, a dentist--he serves on a committee that can actually move legislation forward and I know in my discussions with him last evening he is anticipating putting together some legislative fixes to this problem. He does so not just from a legislative standpoint, he is not an attorney, but he has been an expert in medical practice here in dentistry, so he knows a lot of what he is talking about, has a very great deal of experience that we can draw upon.

And I am also, pleased that Rick Keller--there are four Members of Congress that share Winter Park. It is a great community to share, but I am pleased that he came out. He shares my concern about what is happening in our community, again across the State, and Nation with abuse of prescription medication, so this is an important area.

I was sitting here thinking, as we convened the hearing, back to I think it was December 1980, Senator Paula Hawkins was sworn in this room in advance actually of her term. It was a prearranged swearing in so she could gain a little bit of seniority, and she really began some of the fight to address the problem of illegal narcotics, bring it the attention of the U.S. Senate, the Congress, the problems we had back in the 1980's. At that time it was cocaine and other drugs.

And so, it is ironic that we are back here.

When I took over chairing this subcommittee--but before that when I was on the committee, Mr. Hastert--Mr. Souder served with and got to know the current Speaker very well in service. He was very dedicated to addressing the problem of illegal narcotics, and we conducted back in the late 1990's a hearing in Lake Mary on the problem of heroin addiction. I point that out because we continue to be challenged as a community, State, and Nation on the problem of illegal narcotics. Some of that now has shifted to abuse of prescription medication, and particular, again the focus of this hearing is OxyContin.

For the record, Mr. Chairman, we did a little review of some of the statistics, back in 1999, we had in central Florida 80 heroin deaths, and that was considered an epidemic. In

2000--and actually we had zero according to the figures I have of OxyContin deaths, overdose or deaths from OxyContin. In 2002, we had 68 deaths in central Florida. If we look at it statewide, in 1999, we had 198 heroin deaths, had zero that I have a record of for OxyContin. In 2002 we had 589 OxyContin deaths, as opposed to 326 statewide for heroin. So, if we had a serious problem or epidemic then, we certainly have a situation that deserves our attention as an oversight committee, today.

Finally, I want to say that the purpose of this hearing is to find some positive solutions to deal not only with one particular drug, but any drugs, whether they are illegal or legal, find means and ways of keeping them out of the hands of people who abuse them, misuse them. In some cases we find they are stealing, robbing, pillaging to obtain those narcotics. It is our responsibility in Congress to make certain that we have adequate legislative and law enforcement and agency rules, regulations and laws, to deal with a problem of this magnitude. So, I am hopeful that this hearing will help us find some positive solutions.

I look forward to my colleague, Mr. Norwood, Dr. Norwood's legislative proposal. I look forward to hearing the testimony today from, of course, members of the community who have been affected by the ravages of misuse of prescription medication. We look forward to hearing from some of the national experts, that have been assembled here in Winter Park. And I think that we will also, hear from our law enforcement folks who had to deal with some of the problems created by misuse, abuse, addiction to prescription medication.

So, again I welcome Chairman Souder, I thank you, and again I hope we can have some positive results from this oversight hearing. I yield back.

Mr. Souder. Thank you, I would now like to recognize my friend, Congressman Norwood. When we first ran in 1994, both of us, I as a small businessman, and he as a dentist, we never thought we were going to be Congressmen. And then we came in this big wave and all of a sudden over the years it has been developed that we are in the majority, and we not only have the Senate and the Presidency, and it is a whole lot different now actually with the responsibility of having to figure out how to do these things and work them out. But, it has been a great opportunity to work together and join our other colleagues, and it is great that you could be here today.

Dr. Norwood. Thank you, Chairman Souder, for allowing me to join you today. As you know we have great interest in this subject in the Health and Environment Subcommittee out of the Commerce Committee, and I am grateful for the opportunity to listen and learn today.

I also want to thank my host Mr. Mica, for the hospitality that he has shown me during this visit. I will tell you it is unusual for Georgians to say nice things about Floridians this close to football season, but I do appreciate the warm welcome and I have enjoyed being in your hometown.

The use of drugs to relieve pain is a subject which I have had significant experience in my life. I have experienced it when I was in Vietnam treating wounded soldiers. I have experienced it as a practicing dentist for 25 years. I have experienced it with family and friends through difficulties they may have faced in life, and I have experience a little bit of it personally after a car wreck in 2000.

I feel pretty strongly that we do not do a good enough job

to alleviate pain when we can, and morally and ethically we should. I will say I think we are doing a much better job of that today, then we did in the 1970's and 1980's. I also know that drugs that relieve the most severe pain can be those drugs that are most dangerous. The value of drugs in relieving pain is obviously a double-edged sword. These drugs can create a dependency that makes it difficult for sufferers to wean themselves off those pain killers, and these pain-killing drugs can be diverted for recreational use by abusers. That is actually why we have the Controlled Substance Act, that is why we hold certain drugs to be in a higher regulatory standard, because we are concerned about how they might be used or abused.

I come to this subject knowing that OxyContin has been controversial because of abuse and misuse and diversions of the drug, and I strongly believe we should work to eliminate the abuse of OxyContin and we will. But, I also believe we should work to eliminate the abuse of all controlled substances, it is not the only one that is addicting, and it is not the only one that is dangerous. But how we do this is critical. If we come up with solutions that discourage our physicians from prescribing appropriate pain killers, pain care in this country will take a serious step backward. And we all must remember unless you have been there, unless you have had that pain and can hardly live with it, you do not understand personally the importance of what these drugs can do for you.

I believe there are several areas we need to address if we are going to attack prescription drug abuse and Lord knows we need to. I support the use of state-based prescription monitoring programs. My friend Congressman Chairman Harold Rogers has been funding an appropriation that allows States to set up these monitoring programs, and they are out there in 18 States. With a monitoring program, a State could then catch a person who is running from pharmacy to pharmacy getting a prescription filled. The State could also raise questions about doctors who appear to be illegitimately writing controlled substance prescriptions and my view is that if they are and they are caught, they ought to be put under the jail. That is where one of the problems is.

Today, there is little in place in this country to stop either of these abuses. I come from the time even in the 1980's where we had to keep our prescription pads under lock and key, because people actually would come into the office for bogus reasons hoping that I would walk out of the room where they could grab a pad. I believe we need to reign in Internet pharmacies. That may be the greatest danger. Right now I could go on the Internet and buy a controlled substance just by pointing and clicking two things, I need the drug and I am not lying. So could my 13 year old granddaughter. There are legitimate Internet pharmacists, but those that do not require prescription from a treating provider are going to have to change the way they do business. That loophole must be closed.

When a drug leaves a manufacturer, where does it go? The more I learn, the more concerns I have that our systems have giant holes that allow counterfeit drugs to enter the system. Last year, there was a counterfeit Lipitor scare right here in Florida. That made it much more difficult for wholesalers in this State to sell drugs without knowing where they came from, and it should be done. Right now, you can go back and forth across the borders of this country with 50 doses of a



prescription. It is called the personal use exemption. However, the law allows you to cross the borders as many times as you want to a day with 50 doses. That loophole has to be closed.

Finally, I want to say a word about OxyContin. OxyContin has a legitimate use for patients in severe pain that I believe must be preserved. And there are other drugs out there that may work just as well. If we banned OxyContin tomorrow, and forbade every drug manufacturer from marketing to doctors, does anybody in this room really believe that prescription drug abuse will go away? It will not, it was there before OxyContin ever came on the market. Prescription drug abuse is bigger than any drug, and it is not caused necessarily by marketing practices. I have an hour's worth of reasoning behind that, but I will not do it, Mr. Chairman, right now. What we need to do is close the loopholes that are in our system.

I thank the chairman and Congressman Mica for allowing me to be here today. I really look forward to hearing the testimony of the witnesses. This is a real learning effort for my subcommittee, and I am grateful to both of you. Thank you and I yield back.

Mr. Souder. Thank you. And right now I would like to recognize Congressman Keller, many of us were very thrilled to see him win his first primary and get elected and become an active Member of Congress, and it is great to be here in central Florida.

Mr. Keller. Well, thank you very much, Mr. Chairman. First and foremost, I would like to thank my colleague from Winter Park, Congressman Mica, for his leadership on this issue, and bringing this congressional field hearing right here to Winter Park, FL. It would not have happened without his leadership, and we certainly thank him.

Also, because of our lax immigration laws here in Florida, a couple of out-of-state Congressman were able to slip through our porous borders and come here today. Chairman Norwood and Chairman Souder, traveled hundreds of miles to be here and that is just a testament to how important this issue is to them. We are very lucky, actually we have three subcommittee chairman up here so some powerful Members of Congress with the ability not only to listen today and learn what the challenges are but, to go back to Washington and do something about it. So, I am just thrilled that they are here in person in our community.

As a member of the Crime Subcommittee in Congress, national drug control policy is something that is near and dear to my heart, and I have to tell you in the interest of straight talk, the abuse of prescription drugs like OxyContin presents some very special problems for Members of Congress like me. On the one hand, these are very powerful and dangerous drugs with as high a capacity for addiction as heroin and crack cocaine. On the other hand, these drugs have legitimate medical uses and may give the only possibility for relief for millions of patients suffering from chronic pain, especially those with terminal cancer, and so we have to listen today, and try to get it in the strike zone and do what is appropriate, and that is why we are here.

And I want to thank you all so much for being here as well. Mr. Chairman, with that I will yield back.

Mr. Souder. I thank each of you for your statements.

A couple of orders of business first. I ask unanimous consent that all Members have 5 legislative days which is basically a week to submit written statements and questions for

the hearing record and any answers to written questions provided by the witnesses also be included in the record. Without objection, so ordered.

Second, I ask unanimous consent that all Members present be permitted to participate in the hearing. Without objection, so ordered.

Let me explain a little bit first about how we conduct our hearings. This is a Federal oversight hearing, it is not a town meeting and it is not like a State hearing where people can testify. It is only invited witnesses, and that others may submit written testimony. So you can submit any written testimony either to Congressman Mica's office or Congressman Keller. And when I asked unanimous consent that all Members have 5 legislative days to submit written statements, which is effectively a week, that means it can go through their office. We do not take testimony from the floor. As has been explained several times this is an oversight committee.

In 1994, when we first took over Congress this committee was probably the most high profile in Congress. We did every thing from the Waco hearings to the White House investigations on who hired who, the travel office, China, the FBI files and so on. And so, all witnesses are sworn in. It is one of the only--this is not an intimidation but it is a fact--it is the only committee in Congress where people who have testified have been prosecuted for perjury. Because it is an oversight committee, the statements are presumed to be accurate, so we encourage you to qualify if you are not absolutely certain, because this is an investigative committee.

The name of this Subcommittee is Criminal Justice, Drug Policy and Human Resources. We have jurisdiction over any drug policy and we do authorizing on narcotics issues as well, but because of the nature of how Congressman Mica and Congressman Hastert pulled together these agencies, we also have jurisdiction over HHS and FDA. And we do hearings as well on those subjects and the Department of Justice which includes DEA. And so we are, for example the only committee in Congress, that in addition to drug policy has oversight over both of those different areas, and so we can blend and do followup with both levels of agencies unlike a health committee that can only deal with FDA, or a judiciary committee that can only deal with the Justice Department.

We do different field hearings like this as well in Washington. This subject is not unrelated to others that we have held on illegal narcotics and the difficulty of sorting these things through, but is actually the first one I believe on OxyContin directly. And it is obviously being very closely watched and it is a great privilege to be here in Florida with this hearing. I would like to yield to Mr. Mica.

Mr. Mica. Mr. Chairman, just a housekeeping point. I think Members are aware last week of the ricin scare that we had. They did come and collect our mail and also some of the mail delivery has been suspended. I have had an extraordinary number of request to submit testimony for the record and the chairman is leaving the record open for 5 days. However, I would advise those who want testimony submitted either to get it to Congressman Keller's office, hand carried to Congressman Keller's district office, or my district office. We will be glad to make certain that it gets to the subcommittee within the required amount of time. And I am not sure how you are accepting mail, whether we need an offsite location. Maybe by

the end of the hearing, we can make certain that we have a location. There may be some delay in the subcommittee or Members of Congress receiving that testimony and that does give me some concerns, so we can look into that and, also I think the chairman is going to announce a fax number if you want to submit for the record.

While everyone cannot be a witnesses in these formal congressional hearings, they do have an opportunity to submit for the record testimony.

Mr. Souder. I thank the chairman, that was a good point over the mail. We do not know how much mail, it is not the first time and the procedures sometimes take forever to get to us. The best way is not to send written materials to our offices. Either our fax number for the committee is 202-225-1154. The safest thing is to get it to a Member's district office here in Florida.

With that, we would like the first panel to come forward. Mr. Terry Fernandez of the Central Florida HIDTA; Dr. Robert Meyer, of FDA; and Mr. Tom Raffanello, of the DEA. If you could come forward and remain standing. Will you raise your right hands.

[Witnesses sworn.]

Mr. Souder. Let the record show that each of the witnesses responded in the affirmative.

For those who are not familiar we have a 5-minute clock, so we have time for questioning. It will turn yellow after 4 minutes. We will be a little flexible with that, but to make sure we have time for questioning and get all our panels in, we ask you that all written statements will be submitted. Any additional material be submitted. So if you want to summarize-- however you want to do this is fine. Mr. Fernandez, you are recognized first.

STATEMENTS OF WILLIAM T. FERNANDEZ, DIRECTOR OF CENTRAL FLORIDA HIGH INTENSITY DRUG TRAFFICKING AREA, OFFICE OF NATIONAL DRUG CONTROL POLICY; ROBERT J. MEYER, M.D., DIRECTOR, OFFICE OF DRUG EVALUATION II, CENTER FOR DRUG EVALUATION AND RESEARCH, U.S. FOOD AND DRUG ADMINISTRATION; AND TOM RAFFANELLO, SPECIAL AGENT IN CHARGE, MIAMI DIVISION, DRUG ENFORCEMENT ADMINISTRATION

Mr. Fernandez. I would like thank the Chair and the committee for the ability to be here today, and I would like to thank you for your efforts in this effort--and in this field.

The State of Florida has seen an alarming increase in the abuse of pharmaceutical drugs in recent years. Most specifically OxyContin, and others that contain its active ingredient, Oxycodone. The Controlled Substances Act has placed Oxycodone under Schedule II due to its highly addictive potential.

OxyContin is a drug with two identities--an FDA approved schedule II drug developed for treatment of long term moderate to severe pain, and a substance that can be used by the heroin addict due to its similar euphoric effect. OxyContin also provides the heroin user with the security of a predictable potency in a regulated dosage unit. There are instances of the OxyContin abuser switching to heroin in some parts of the State.

Abusing an OxyContin tablet is easily accomplished by chewing the tablet thereby voiding its controlled-release

feature. The tablet can be crushed and snorted, or made soluble and injected. It is often mixed with other licit and illicit drugs which can prove very deadly.

In 2002, there were 589 drug deaths in the State of Florida in which Oxycodone was found in the system. Oxycodone was found in lethal amounts in 256 of these. During the first 6 months of 2003, there were 292 deaths involving Oxycodone. It was found in fatal amounts in 136 persons, 48 of whom were central Florida residents. Of the 136 Oxycodone fatalities in the first half of 2003, 67 percent were over the age of 35 and 16 percent were over the age of 50.

Intelligence indicates doctor shopping, prescription fraud, and robbery, are the three most common means of obtaining OxyContin.

The heroin problem in central Florida has certainly contributed to the abuse of OxyContin and other drugs containing Oxycodone. Further, the lack of availability or increase in price of one, motivates the abuser to seek the other.

I cannot recall a substance so diversely abused, crossing all age groups, ethnicities and social statuses, with such a devastating effect. We know the source of this drug, the retail price, the illicit price, the distribution routes, and very much about the end user and his supplier.

I refer to the November 2003 article in the South Florida Sun-Sentinel which lists the top 12 OxyContin prescribers for Medicaid during the period 2000 to 2002. These 12 doctors wrote prescriptions totaling \$15,645,745.00. This figure represents 1,689,605 80-milligram tablets of OxyContin or 9,540,000 10-milligram tablets. Should our efforts to bring this abuse under control not start here?

The Florida Prescription Validation Program utilizing an electronic data base containing prescription history and counterfeit-proof prescription forms will certainly assist in curbing doctor shopping and forged prescriptions.

The validation program in cooperation with the Drug Enforcement Administration's Office of Diversion Control and its registry of physicians prescribing controlled substances, should be a natural alliance.

Thank you.

Mr. Souder. Thank you, and I should have repeated that Mr. Fernandez is the director of the Central Florida High Intensity Drug Trafficking Area, Office of National Drug Control Policy, which coordinates State, local and Federal anti-drug efforts in central Florida.

Now we are going to hear from Dr. Robert Meyer, Director of Office of Drug Evaluation II, I should have said earlier, the Center for Drug Evaluation and Research, of the U.S. Food and Drug Administration, FDA. Thank you for coming.

[The prepared statement of Mr. Fernandez follows:]

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Dr. Meyer. Thank you. Good morning Mr. Chairman and members of the subcommittee. I oversee the review division that has regulatory responsibility for the high dose of opiate analgesic products. And I appreciate the opportunity to talk about FDA's drug approval process and our role in preventing prescription drug abuse.

FDA is a public health agency that is strongly committed to promoting and protecting the public health by assuring that safe and effective drugs are available to the public. FDA is aware of and is concerned about reports of the growing problem with prescription drug abuse. We understand the seriousness of this issue and sympathize with the families and friends of individuals who tragically lost their lives or otherwise have been harmed, as a result of prescription drug abuse and misuse, including OxyContin.

We also sympathize with the many pain patients who suffer needlessly due to under treatment or substandard treatment. In taking actions on these matters, FDA must strike a critical balance.

Let me turn for a moment to one of the issues upon which I was asked to speak, the FDA drug approval process. Under the Food, Drug, and Cosmetic Act, FDA is responsible for ensuring that all new drugs are safe and effective. Before any drug is approved for marketing in the United States, FDA must decide whether the studies and other information submitted by the drug's sponsors have adequately demonstrated that the drug is safe and effective when used according to the drug's labeling. When the benefits of a drug are found to outweigh the risk, and the labeling instructions allow for safe and effective use, FDA approves the drug for marketing.

There are instances where FDA may develop, in cooperation with the drug sponsor, a plan of intervention beyond just labeling to help assure the safe and effective use of a drug. This has recently been referred to as risk management plans [RMP], but the practice dates back many years. These interventions making up an RMP may be varied but all are aimed at assuring that some known or potential issue regarding the proper use of the drug is addressed when the drug is used.

During the approval process, FDA assesses a drug product's potential for abuse. If a potential for abuse is found to exist, the product's sponsor is required to provide FDA with all data pertinent to the abuse of the drug, a proposal for scheduling the drug under the Controlled Substances Act and data on overdoses. Under the Controlled Substances Act, FDA must notify DEA if a new drug application is submitted for any drug that is assumed to have abuse potential, and that includes depressants, hallucinogenics, or stimulants.

Finally, it is important to state that FDA's job is not over when the drug is approved. The FDA conducts post-marketing surveillance that monitors drugs post-approval for their safety, allowing for reassessments of drug risk based on new data learned after marketing. When needed, we then recommend ways to most appropriately manage these newly identified risks. In part prompted by our experience with OxyContin post-marketing, FDA has undertaken a number of actions to help prevent prescription drug abuse.

First amongst these is FDA's actions and planned actions with the regard to drug labeling of the high dose opiates, particularly the extended release products. Labeling not only serves as an important means of informing prescribers and patients about the proper use of a drug, but also importantly defines the bounds of marketing and advertising for that drug. Labeling to these opiate products should emphasize that drug treatment for pain should be initiated at a level appropriate to the pain and condition of the patient.

Additionally, labeling should help prescribers properly

assess potential patients for the likelihood of abuse. In particular, patients with a personal history of substance abuse or a strong family history of abuse should be considered as being at higher risk for drug abuse. It should be noted that when significant changes are made to a drug's labeling, FDA encourages the drug sponsors to notify health care professionals, and to educate them about the serious risks. And FDA helps in the dissemination of this information via its Med Watch program and its Web page, amongst other means.

A second important means by which FDA addresses issues of drug abuse is through the regulation of prescription drug marketing.

A third way that FDA can use to address these problems is through the development of risk management plans as I mentioned earlier.

A fourth means that FDA uses to meet this challenge is by working with other involved entities, such as government agencies, industry and professional groups. We work with them to share information and insights needed to address this broad problem. For instance, FDA and DEA meet regularly to discuss ways to prevent prescription drug abuse and diversion, and we are working on the following areas with DEA: physician education, State prescription drug monitoring programs, a joint task force participation focused on illegal sale of controlled substances, and the assessment of new products with abuse potential.

In conclusion, FDA recognizes the serious problem of prescription drug abuse. The agency has taken many steps to address this serious problem and will continue to act to curb abuse, misuse, and diversion. Since this problem is broad in its scope and implications, we are committed to working with our partners. We share the subcommittee's interest and concerns regarding prescription drug abuse and would be happy to answer any questions.

Thank you.

Mr. Souder. Thank you. We will now hear from Mr. Tom Raffanello, Special Agent in Charge, Miami Division, Drug Enforcement Administration. Thank you for coming today.

[The prepared statement of Dr. Meyer follows:]

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Mr. Raffanello. I am here before you today to discuss the challenge of prescription drug abuse, and the efforts of the DEA to combat it. My name is Tom Raffanello, I am the Special Agent in Charge of DEA's Miami Field Division, which is the entire State of Florida.

I would like to thank this subcommittee on behalf of Administrator Tandy for your unwavering support of the men and women of the Drug Enforcement Administration and its mission.

Opiates in pill form have historically been among the most abused prescription drug, especially hydrocodone, hydromorphone and oxycodone. Diverted from legitimate channels these drugs can substitute for illegal narcotics and are frequently trafficked on the street by individuals or structured organizations. As far back as the 1970's, hydromorphone based Dilaudid was known on the street as drugstore heroin. Prescription drug abuse has recently escalated to a new level of concern with the development of opiate-based pain killers designed for controlled or sustained release. These products pose special challenges to law enforcement. It is easy to see why when you consider OxyContin contains 2 to 16 times the dosage of oxycodone as its well known predecessor Precodan.

OxyContin is also the most widely known example of an abused prescription drug, and its diversion has increased dramatically since its introduction into the market. OxyContin is a valuable and efficient pain management drug when properly prescribed and used. At the same time, however, its popularity for abuse sky-rocketed when word made its way to the street that manipulating this powerful drug can bring heroin-like effects. DEA has never witnessed such a rapid increase in the abuse and diversion of a pharmaceutical drug product.

The popularity of OxyContin and other drugs of abuse have also inspired a wide range of diversion methods, some new and some old. Practitioners and pharmacists illegally or indiscriminately prescribe or dispense OxyContin for profit. Addicts and dealers steal drugs through pharmacy thefts and in-transit highjacking. Forged or fraudulent prescriptions are common occurrences as are patients who claim false medical needs. Doctor shopping abusers travel from doctor to doctor to find an easy mark who will readily write prescriptions or who can be duped.

Foreign diversion and smuggling of contraband drugs into

the United States continues to be a problem. Perhaps the greatest concern, the Internet, has become a virtual wild west bazaar for spam e-mails and Web site advertisement that sell controlled substances with little or no oversight that the drugs are sold for legitimate medical reasons.

At times, multiple methods of diversions occur simultaneously. In Sarasota, FL, a physician recently was arrested for writing prescriptions for controlled substances to known drug dealers and abusers including Dilaudid and OxyContin. The doctor saw as many as 80 patients daily, charged \$250 for an initial office visit and \$150 for followup appointments. During the search of the physician's office, DEA and local law enforcement seized approximately 25,000 dosages of controlled substances including large quantities of oxycodone, methadone, and hydrocodone.

In response to growing concern among Federal, State, and local officials about the dramatic increase in the illicit availability and abuse of OxyContin, the DEA initiated an OxyContin action plan in May 2001 as a comprehensive effort to prevent diversion and abuse of the drug. This is the first time the DEA has taken such a comprehensive approach to a particular brand name prescription drug. The initiative is not intended to impact the availability of OxyContin for legitimate medical use.

The plan has four main goals: First, to enhance the coordination of enforcement and intelligence programs with other Federal, State, and local agencies to target individuals and organizations involved in the illegal sale and abuse of OxyContin.

Second, to use the full range of regulatory and administrative authorities to make it more difficult for abusers to obtain OxyContin. The DEA does this by closely monitoring the quota of oxycodone available to manufacturers, continue to work with the FDA to reduce the abuse of reformulated OxyContin by injection, and to continue our efforts to improve physician education on treatment of pain and recognition of addiction.

Third, increase the cooperative efforts with the pharmaceutical industry.

Fourth, advanced national outreach to educate the public, the health care industry, the schools, and the State, and local governments on the dangers related to abuse and diversion of OxyContin.

DEA is also, working with States on prescription monitoring programs, to prevent diversion at the State level. PMPs capture information regarding prescriptions electronically at the point of sale, usually the pharmacy. The information is transmitted to a State agency to identify the doctor shoppers, and/or other evidence of diversion. Sixteen States have activated PMPs and another five States have partial or pending programs. The General Accounting Office concluded in a 2002 study that PMPs have aided investigators and helped to reduce doctor shopping.

For the past 2 years, Congress has appropriated funds for States to initiate and expand PMPs. Florida has applied for an enhance grant of \$350,000 to augment an initial grant beginning in January 2005.

Mr. Souder. Mr. Raffanello, if you could kind of summarize. Mr. Raffanello. Surely.

The DEA is committed to protecting the American public's health and safety from the serious consequences of abuse of



legal pain relief for life destroying illegal purposes.

Initiatives like the OxyContin action plan, PMPs and additional diversion investigators to be able to work on the Internet abuse that we have will help the enforcement effort that we feel is the key into slowing down and doing with the problem.

I thank you very much, and I will answer any questions that you gentlemen have.

[The prepared statement of Mr. Raffanello follows:]

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Mr. Souder. Thank you.

Let me start with Dr. Meyer. You said in your testimony that the FDA did not anticipate what was going to happen when you first cleared this OxyContin. Do you seek input from DEA and all the anti-narcotic agencies when you are clearing it?

Dr. Meyer. We notify DEA, of the fact that we have the NDA in house and we work with DEA on establishing a quota for the drug substance that goes into the drug product.

Mr. Souder. Do you believe that the actions, because you gave me a list of actions that you have done since then because, according to your testimony, as the abuse spread, FDA then changed labeling, and you have been trying to catch up. Do you believe had you done all those things at the beginning, we would not have this problem or do you believe that the things that you are doing are not effective in stopping the problem unless something else is done?

Dr. Meyer. I think that the things that we have done will have an effect and I think if we had put them in place at the beginning, that we would have less of a problem than what we have now, but I think the problem goes beyond the means available to the FDA, or beyond this particular drug.

Mr. Souder. Mr. Raffanello, you stated that there has not been another prescription drug abused at this level?

Mr. Raffanello. That is correct.

Mr. Souder. Anything even approximating?

Mr. Raffanello. I believe Dilaudid for many, many years has been used as a heroin substitute, and very effective.

Mr. Souder. What would you have done differently at the beginning, and as we look at other similar things possibly coming on the market, because at this point if OxyContin went off something else would likely come on. What would you do different at the very beginning in addition to some of the things I think we are trying to address now, because once it starts to explode, it is just so hard to control it?

Mr. Raffanello. Being a career law enforcement officer, I would make sure that practitioners and pharmacists knew that there would be a penalty to pay for over-prescribing or for doing anything that even smites of going against the law. I think strong law enforcement would be a key.

Mr. Souder. Mr. Fernandez, you stated in your testimony that you would first look at--which is kind of a logical business approach--at the top 10 people who are currently prescribing it. Is that not being done? It does not mean that they are doing it illegally, but why would that not be the first place you would look? I think your testimony said that there were the top 12 OxyContin prescribers for Medicaid, the 12 doctors that wrote prescriptions this figure represents so much, should our efforts to bring this abuse under control not start here? Why would it not start here, what is keeping it from starting there?

Mr. Fernandez. I do not know that anything is keeping it from being started. I think I made the statement basically to show you--I mean, to me it is just inconceivable that 12 doctors wrote prescriptions totaling that much. And that is just Medicaid. I do not know how many more they wrote that had nothing to do with Medicaid.

Mr. Souder. You coordinate the Central Florida HIDTA, Mr. Raffanello is the Miami DEA person, we have the representative from FDA. I would like to know why would it not start there, and why has nobody started there?

Mr. Fernandez. I do not think it is--and I could be wrong here, but I do not believe anyone knows when they are writing it until after the fact, and then it is too late. That is why at the end of mine, I recommended this the Florida----

Mr. Souder. Who has jurisdiction to start that? Would that be an FDA responsibility to look at that currently, and say we have 12 doctors who wrote this many? Here is what it seems like. I am a Member of Congress, and this is still what it seems like. I thought it would be different after I got out of the private sector into the public sector, that when we go after hospitals in the United States for Medicaid or Medicare or whatever, it seems like we take the ones that are easy pickings off the tree who are filing all the stuff and we get somebody who has 2 percent of the market and skip the people who have 90 percent of the market. It does not mean that these 12 doctors are doing anything wrong, but why would that not be the first place you look to check and see what the failures of the system are? Have those people been looked at and who would be responsible for that, I do not understand here?

Mr. Raffanello. Maybe I can help. The Drug Enforcement Administration has a diversion responsibility, and in that responsibility we monitor practitioners and doctors. I would--unfortunately I would like to go back to the times that Congressman Norwood said, when they were kept under lock and key. It is one thing to be able to look and see a pattern, it is another thing to effect an arrest, and get someone to prosecute the case. It is very, very difficult to prosecute and convict a doctor or a prescriber for one of these types of offenses initially. But in my opinion, that is where the work really needs to be done. If people that prescribe this knew there was a severe penalty to pay, you would have less people doing it. And that is where we should start.

Mr. Souder. Mr. Mica.

Mr. Mica. Let me just continue along the line of questioning of Chairman Souder. It is difficult to convict--where is the flaw, is it in the Federal law, is it in the FDA regulations of the narcotic? What is the problem?

Mr. Raffanello. I think it is a fairly new phenomenon. I think that the States may in some cases not have the law. We

have--in Federal statutes, we can take the doctor's license away. Criminal statutes are always the last resort. I would like to see prosecutors more energized to pursue criminal statutes, I would like to see the States work through their legislation to have severe criminal penalties for doctors, for pharmacists, and for people that prescribe it. I think that the groundwork is there, I just do not think we have them to the level that we need to have them to make the impact that we want to have.

Mr. Mica. Is this something we need to do from a Federal level or State by State? I mean, it does not sound like we can get a handle on it if we rely on 50 legislatures to act.

Mr. Raffanello. Speaking from the Florida situation, we are very fortunate here. I work with Jim McDonough, the director of the Florida Office for Drug Control, and they aggressively pursue this in the State of Florida.

Mr. Mica. But, again, OK that is a State agency, we have the HIDTA which does the combination State, Federal, all efforts, you are DEA, Federal. Do you have enough laws and tools to deal with this? You also testified and we heard similar testimony about diversion, about Internet access. Chairman Norwood said that his young 13 year old could get this stuff in quantities. We need to know where the gaps are and if they are Federal gaps we need to know that, and particularly from you. So you are recommending tightening one, two, three, tell us?

Mr. Raffanello. At first I would look--the Internet has been a tremendous source for drug distribution. I would go back and see what we have. If someone in Oklahoma, took applications and prescribed drugs in Florida, they should be able to be tried in whatever district is affected. I believe that the law on that is very vague right now.

Mr. Mica. Right. Now, the other thing you have is people becoming addicted to a legal source of the prescription and then the second part is illegal availability through prescription fraud. You described prescription fraud. How do we address that from a Federal standpoint? Those two.

Mr. Raffanello. I would go back to more inspections on doctors and pharmacists, and tighter reins on just what they are doing. I think the prescription program that we are now trying to work with Congressman Rogers' help would be something that I would like to see supported, so we can automatically see who is being prescribed. I think we have mechanisms that need to be tightened up there, and need to be applied across more States.

Mr. Mica. All right. FDA.

Dr. Meyer. Yes, sir.

Mr. Mica. Abuse of narcotics as we have heard, I gave this historical sequencing, starting in this room with election of Senator Hawkins, the cocaine problem, the heroin problem. Of course, we have cited here a different prescription drug problem and this is now a prescription drug of choice that we found being diverted. Has FDA adequately changed its rules, its regulations regarding abuse and misuse of this substance?

Dr. Meyer. I think that a lot of the abuse and misuse is occurring in circumstances where FDA does not actually have strict purview. I think our main----

Mr. Mica. So does the law need to be tightened to give you that purview?

Dr. Meyer. I think I would defer to DEA, since DEA has the

jurisdiction on this, whether they would need something, but, FDA does not regulate the practice of medicine. Much of this is occurring in the setting of----

Mr. Mica. Well, you discovered a drug where we have deaths off the chart here that doctors are--and we have had testimony here of 12 doctors on Medicaid issuing incredible volumes of this stuff and people are dying in an unprecedented numbers. So you either you change the rules or we change the laws, and if we need to change the law, do we have enough laws directing FDA to deal with this or do you already have that authority?

Dr. Meyer. Again, I do not believe we have the authority to act with regard to how these drugs are used in the practice of medicine.

Mr. Mica. All right, I want you to submit to me a written statement. You can do it through the committee of what it would take for you to have the authority under the law to more aggressively pursue this matter, can you do that?

Dr. Meyer. We can do that, be happy to do that.

Mr. Mica. As an agency--and I would like you to submit the same thing to me as far as any loopholes or changes that DEA sees--our enforcement agency--so we have a better handle on how we can change the law. You have the ability to change regulations already within the law, so I need to know specifically what we can do.

Mr. Fernandez, you talked a little bit about electronic data validation, the problem with getting a handle on people who are prescription shopping and I was interested in that. Could you elaborate a little bit more how we get a handle on medications, not just OxyContin, but drugs that can be used, prescription drugs that can be used and abused, and how do we get a better handle on all of this?

Mr. Fernandez. Yes, sir. I think there is a gap between the doctor writing the prescription and the people that give that doctor the ability to write that prescription. I do not think the Federal level gets the information as rapidly. I do not know if they get it at all in some cases, but I certainly do not think they get it in a timely manner. That was one reason I referred to the Florida prescription validation program. And I do not claim to be an expert on that. Mr. McDonough can certainly tell you more about that than I could. But, as I understand it, a prescription would be written and it would be computerized and State officials would know. I would assume then they would see a doctor writing more than he should be.

Mr. Mica. It disturbs me when we have a Federal program and you cited, right, 12 Medicaid doctors?

Mr. Fernandez. I got that from a newspaper article; yes, sir; 12 doctors wrote prescriptions totaling over \$15 million.

Mr. Mica. So, a Federal program they are gaming to bring on the market, a substance of which hundreds of our people are dying. Well, I would like to--Mr. Chairman, I did have an opportunity to meet with some folks I believe that are involved in this electronic data validation program under Medicaid, which I believe the feds and also, the State is supporting. I would like to ask unanimous consent to submit for the record testimony by Jim Kragh who is the president of Good Health Networking. He demonstrated to me I guess this is just a little type of a Palm Pilot. But the software does electronically validate prescriptions, gives us a better handle on what prescriptions and what amounts, and who the users are.

So, I would like Mr. Kragh's testimony to be submitted as

part of the record, describing what I understand in central Florida we have over 800 physicians participating in this demo to get a handle on where these prescriptions are written. So I ask unanimous consent for that submission.

Mr. Souder. Without objection, so ordered.

[The information referred to follows:]

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Mr. Souder. Mr. Norwood.

Dr. Norwood. Thank you, Mr. Chairman, I appreciate it. Let me, I would be remiss if I did not introduce you to our court reporter, Bill Warren. He is my only voter in this room, and I am glad Bill is here from Monroe, GA.

Mr. Fernandez, you said in your report, intelligence indicates doctor shopping, prescription fraud and robbery are the three most common means. Just so you and I are on the same page, define doctor shopping?

Mr. Fernandez. Going to one doctor and getting as much as you could on a script, claiming to have pain, going to a second doctor within probably a short period of time, getting another prescription written for the same.

Dr. Norwood. I would define it the same way, I just was trying to see if you meant by that going to 1 of the 12 who the underground knows is writing bogus prescriptions. Is that

doctor shopping, too?

Mr. Fernandez. Yes, sir.

Dr. Norwood. Just to make the record clear there are thousands and thousands of physicians in Florida that take Medicaid, and the fact that we are talking about 12 makes me wonder why we could not deal with that 12. You obviously have information, do you have that information, on those 12?

Mr. Raffanello. I am unfamiliar with it.

Dr. Norwood. Why would you know that, and he not, because any time 12 physicians are writing \$15 million worth of Schedule II drugs, my dander goes up a little bit. Something does not smell right about that immediately, and I would think somebody ought to be asking those people some questions. What is going on with that 12? Recognizing what you said is so true, this is very difficult, I will get into that in a minute.

Mr. Fernandez. I got the information from a newspaper article that I referred to earlier, but they named the doctors, there were 12 doctors.

Dr. Norwood. So we know who perhaps--through Medicaid records, who these people are?

Mr. Fernandez. Oh, without a doubt, and all of these doctors had prior problems, would probably be a good word. I think they have had some run-ins with different medical boards and what-have-you.

Dr. Norwood. Just to make this point too, taking the license is not the solution. Frequently that drives the physicians underground. They do not have a way to make a living, it just gets worse in my opinion. There needs to be criminal activity, as you pointed out, involved in this and the penalty for this needs to be very, very steep. That does not mean maybe the license is not taken and they can practice in prison, but my view is we do not take it seriously enough. I do not know the percentage. I do not know the number of doctors that see Medicaid, but that is a low percentage. At least that 12 ought to be visited on a pretty regular basis.

One of you were pointing out earlier, all of this, these prescriptions, you do not know about it frequently, until after it the fact. And that is what we are looking at in our committee is how you could know it a little sooner. Because there are patterns that occur. I mean, just like this 12, you can know who they are pretty easily. This 12 do not just see Medicaid patients, incidentally.

So you do not really know the numbers until you go in, until this is a criminal problem rather than just a problem with the ethics committee or the State board of dental examiners.

I am glad to hear you talking about the Internet. I wonder--and we are thinking about this too--if we outlawed in this country the purchase of Schedule IIs over the Internet, do any of you know how much might be available out of this country, to buy over the Internet from India? And we do not know how to fix that. We do not know how in the world we would keep it from coming across the Internet if we closed down every Internet prescription shop in America. Any comments, any thoughts?

Mr. Fernandez. I cannot answer that question, but I would ask the good doctor here, we do know who produces OxyContin. Is it just one company?

Dr. Meyer. OxyContin is only produced by one company, but oxycodone has many sources and I do not, as the U.S. regulator

of drugs, we do not have a good handle on, say who might produce it in India, unless it is for a U.S. manufacturer.

Dr. Norwood. Well, this is so profitable even though I do not know where the company is that produces OxyContin, but it is so profitable, if we shut that company, or control them tight, it is going to pop up somewhere else.

Which is part of my point of this hearing. The Dilaudid, look I remember when people were trying to come in and get Dilaudid. Now they want OxyContin. Though I never prescribed OxyContin it seems to work really well for pain relief, which means it also works really well for the people who would abuse it. That is why they want that today, that is the popular one today. It used to be Percocet, Percodan. You guys have been around long enough to remember when that was. But we have to--I think it ought to be a State program, Congressman Mica, maybe under a Federal umbrella, because we have to cross State lines. You have to be able to--if you are going to stop doctor shopping they cannot come to three doctors in Augusta, GA, cross the Savannah River and go to three in Aiken, SC, without us having some handle on that.

Part of the difficulty is how do you do this with privacy. But somewhere, somebody, has to collect this information and it has to be electronic and automatic, they do it now anyway. They immediately send out electronic messages to get paid from the pharmacist. That same message could go to some collecting point, so that we do know and you could know in the State of Florida. You pick up abuses on that in 2 minutes. Do you need more--you need the penalties to be greater, do you not?

Mr. Raffanello. Yes, absolutely. I have just two points I would like to make in response to your question----

Dr. Norwood. Please.

Mr. Raffanello [continuing]. About out-of-the country sources. If past be prologue, in the past when we had problems with Qualudes--and we did a very efficient job in the United States banning them--Mexican traffickers took to taking the precursor chemicals, took to using the pill presses, and did exactly that in Mexico. The Mexican and Colombian traffickers are very, very ingenious. They will fill a void. If there is a need, they will fill a void. I think that eventually they would probably use the Internet or use 2,200 miles of border. They will use whatever they perceive is a weakness to be able to do it. And that is something that could happen in the future and that is something that DEA, intelligence-wise, is looking at.

Second, on 12 doctors with Medicaid, primarily Medicaid fraud is--I believe there are several other Federal agencies that have that initial responsibility. What happens in those scenarios is when they get to the point where they want to pursue a Title 21 offense, then they will call DEA and bring DEA in. But the vast majority of the time, the offense is initially discovered by the agency with the oversight of the doctor on the Medicaid program.

So it is not--I do not have the manpower to be able to cover every doctor in Florida, and DEA does not have that kind of manpower, but the people with Medicaid oversight, if they see something that does not look good, they will often call us. And if it is another Federal agency, they may have Title 21 authority, and they may do it themselves.

Dr. Norwood. Have they called you about these 12 doctors?

Mr. Raffanello. Not at this point.

Dr. Norwood. Then they are not doing their job.

Mr. Raffanello. If it were the Federal Bureau of Investigation, they have concurrent jurisdiction to Title 21. So they may decide to enforce that themselves, that is a possibility. I promise you that I will find out more about it.

Dr. Norwood. Well, in conclusion, Mr. Chairman if I may, we can stop I believe immediately--not immediately but pretty quickly--the three problems you are talking about. Maybe robbery is a different subject. I know one time I caught a fellow trying to abuse the Percodan deal and we took care of that real quick. We had your folks over there immediately, and of course they tried to burn our office down after that, in a few weeks. I am just telling you how bad these people want these drugs, and they will do anything for it.

But I think we can probably stop the problem of doctor shopping, I think we can stop prescription fraud, maybe we can never stop robbery. In the long term, at the end of the day, the real problem for us about people abusing and getting too many Schedule II drugs, is going to be just what we are talking about--it is going to be the Internet, and it is going to be foreign sources. And you guys are really smart and need to help me figure out how to do that.

Thank you, Mr. Chairman.

Mr. Souder. I think this Georgian is downplaying his own smartness. That is, I take it, a southern trait. A very smart man. Mr. Keller.

Mr. Keller. Thank you, Mr. Chairman. Just to followup on something Congressman Norwood was hitting on. Mr. Fernandez we know from your testimony we have 12 physicians who have written over \$15 million worth of Medicaid prescriptions for OxyContin, that is 9.5 million tablets. As of this morning anyway, the south Florida newspaper Sun-Sentinel, has known for 4 months who these people are, but as three Federal experts sit here today, we do not have a clue if they have even been interviewed, any of these doctors by the DEA or FBI.

Mr. Raffanello, what can you tell us about the future prospects with respect to these 12 doctors who have now been identified through public records, and a newspaper; will they at least be interviewed by some law enforcement agency?

Mr. Raffanello. Let me say this, I have 22 officers in the State of Florida, and because I do not know about it, I am assuming the DEA is not part of it. I very well may find out that we are. I have not read the article, I am not familiar with it, I am not familiar with that incident, but all that aside I take responsibility for it, and I assure you that I will find out who has the investigation and they will be talked to.

Mr. Keller. OK, I will tell you, that reminds me, you know, September 11, we had 15 of the 19 highjackers came here from Saudi Arabia. We had one guy at the State Department that issued 10 of those visas. Afterwards nobody talked to him. And I look at this situation--I do not know if we need new laws right now, maybe just some enforcement of the existing ones, and maybe they are being enforced and we just do not know. We have to get to the bottom of that.

Let me ask you a question, Dr. Meyer, are there any specific marketing practices by the distributors of pharmaceuticals that you would like to see stopped with respect to OxyContin?

Dr. Meyer. The FDA has actually found that the vast majority of the marketing of OxyContin specifically has been



within our legal bounds. We have in two incidences cited them for deviating from acceptable practices, going beyond the labeling or not giving sufficient warnings about the misuse and abuse of the drug.

I would say that the company itself has voluntarily elected not to directly market to consumers, and we wholeheartedly agree with that.

Mr. Keller. Does that mean they have not done any TV ads for OxyContin.

Dr. Meyer. They have not done any TV ads.

Mr. Keller. OK.

Dr. Meyer. Right.

Dr. Norwood. Would the gentleman yield?

Mr. Keller. Yes, I will yield, Mr. Norwood.

Dr. Norwood. As long as they do not market to the public, which I would be 100 percent against, and so I understand they are, too. We need to remember who they are marketing to. Actually they are talking to people and trying to encourage them to, and explain their new drug, who should know the pharmacology inside out, who should know the ill effects and particularly the addictive effects, and my view on that is that shame on the doctor who does not explain that to their patient. It is not like they are being talked into using something they do not understand, they do understand, they understand the pharmacology of it.

That is why I said earlier in my opening statement, the marketing to a physician is not abnormal. Most drug companies do want you to use their particular product over another product, but they are not talking to people who totally do not know what they are being asked to use. So, I blame it on the doctor who does not explain it to their patient that we need to be very careful here and monitor that patient.

Dr. Meyer. I would point out, Dr. Norwood, that I think I agree with a lot of what you are saying, pain management has changed greatly in the last 10 to 15 years. When I was licensed in the State of Oregon, we had a mandatory training in pain management prior to getting our license. That was about 12 years ago. A lot of what I was taught then is no longer believed to be true now, so the FDA----

Dr. Norwood. Thank goodness.

Dr. Meyer. Pardon.

Dr. Norwood. Thank goodness.

Dr. Meyer. Thank goodness. I think the FDA in conjunction with DEA and others is supporting better education, because I believe that part of this is education. There is a need for physicians to better understand both the good points of these medicines, how to effectively treat pain, how to screen for abuse and how to help prevent abuse as well. While I think a lot of physicians are very well educated in basic pharmacology, these are specialties or special skills that are not necessarily effectively taught in medical school. So it is really incumbent on us to continue the education efforts.

Mr. Keller. Thank you, Dr. Meyer.

I have one final question for Mr. Fernandez and Mr. Raffanello. The one common denominator from all the questioning from the various Congressman today seems to be that they are very interested in having the Federal Government crack down on the practice of selling OxyContin in similar drugs over the Internet. You seem to have a sympathetic Congress on this issue. Mr. Raffanello, let me start with you. Do you have any

specific steps that you would like the Federal Government to take to crack down on this practice of selling OxyContin over the Internet?

Mr. Raffanello. Yes, and thank you. I would like to do a review and find out what the existing laws are. As I explained before, you will run in to venue problems, prosecutorial venue problems.

Second, that a condition of prescribing some controlled substances that a physical exam be given, you cannot give a physical exam over the Internet. I think we can dispense with a lot of that if we review what we have and let it evolve to take in the fact that it is being exploited by crooks on the Internet.

Mr. Keller. That sounds great, especially a physical exam requirement there. Mr. Fernandez, do you have anything to add to that?

Mr. Fernandez. No, sir, I do not. I think that covers it pretty well.

Mr. Keller. OK, Mr. Chairman, I will yield back.

Mr. Souder. Thanks.

I want to do a couple of followup things to make sure we have these in the record, because we kind of plunged right in with certain implied things. Mr. Fernandez, it seemed from the chart I have heard some of the information that there are more OxyContin deaths than heroin deaths in Florida, at least there were in 2002?

Mr. Fernandez. In central Florida, there were not, there were more heroin deaths. I really cannot speak well for the whole State. I kind of concentrate my efforts for seven counties.

Mr. Souder. OK, let us talk about central Florida for a second. The OxyContin deaths were approximating heroin or far behind? What is the extent of the OxyContin problem here in central Florida?

Mr. Fernandez. It is very bad and growing. And I think Congressman Mica mentioned it earlier, it has happened rapidly and I would like to think that it has peaked, but I do not think it has. Heroin is continuing to grow.

Mr. Souder. Would you compare this to the other threats in the community here from the other narcotics? Is OxyContin, when you get addicted, there are more overdoses and it does not have as much violent crime related to it? Is there a tendency if you get this stolen OxyContin to peddle it, and do you have a dealer network? Or are the doctors in effect who are illegally doing this--give us the social consequence in the community and in hierarchy of trying to decide what your HIDTA focuses on where you see OxyContin?

Mr. Fernandez. My HIDTA is not a good sounding board to be very honest with you. We concentrate on heroin, and we have a DEA led heroin task force that looks at strictly heroin. We have seen surprisingly little OxyContin tablets, we have not seized very many at all. I think it is for a couple of reasons. One I think it is because they come through doctors and the people that we put on the street, our task force do not look there. And I think it has moved in relatively small amounts. And we are constantly encouraging our people to look at organizations and, you know, just bigger distributors.

So far as the addictive abilities and what have you, certainly it is on par with heroin.

Mr. Souder. Let me ask you something, Mr. Raffanello, do

you see OxyContin as a greater problem in other parts of Florida, other than central Florida? I am trying to get a handle on--let me get to my end point here. Why is there not a HIDTA sub-task force on OxyContin, or a DEA task force on OxyContin in Florida that is pursuing this?

Mr. Raffanello. Because, OxyContin--our biggest threat in the State of Florida is heroin, and the heroin deaths exceed the OxyContin deaths. Our second biggest threat is methamphetamine. We have gone from 25 methamphetamine labs several years ago to over 250 this year. We have a different client that uses OxyContin and oxycodone. Unfortunately, sometimes a student or someone will cocktail, will take OxyContin with something else. Most of these oxycodone deaths are not based on oxycodone alone, it is part of what else is in their system.

In the big scheme of things for us, it comes in third in this particular area. And working with the same amount of people we have worked with as agents over the last 10 years, we have to prioritize to our biggest threat. It is not our biggest threat.

Mr. Souder. I cannot remember where I saw it in the materials I was reading for the hearing that I thought it was in Florida that the OxyContin deaths exceeded the heroin. You are saying there are poly drugs?

Mr. Raffanello. That is correct.

Mr. Souder. Are you saying deaths exceed it?

Mr. Raffanello. No, it is not deaths, it is addiction, it is people in emergency rooms. If you just looking for the deaths, I believe my theory is correct, that it is still heroin deaths that, unfortunately, are the No. 1 here. But methamphetamine, because of the endangered children--we are trying to cover all three; oxycodone, at this point is not in their league.

Mr. Souder. So you are saying basically that oxycodone is a danger to the user predominantly?

Mr. Raffanello. Yes.

Mr. Souder. Whereas the difference with meth, even though as many people may not be dying, it is impacting the others in the home more?

Mr. Raffanello. Communities, children, we do not even know what some of those chemicals do to the environment.

Mr. Souder. How many people have to die and at what level does OxyContin have to become a problem here in central Florida, and Florida, before it becomes a part of a HIDTA request or a DEA request?

Mr. Raffanello. Well, that is not our criteria. If we see an emerging trend, and we have, I only have somewhere in the vicinity of 25 diversion investigators for the entire State. And that also includes regulatory functions and that also includes inspection functions. So, quite frankly I am trying to cover a large State with a relatively small amount of people.

Mr. Souder. One of the things, however, it does not prohibit either the HIDTA or the DEA from requesting to headquarters, and then the headquarters can request to Congress and put the blame on us, if we have not funded, which is part of the problem. We have not necessarily funded--we rail against all the different problems and then do not necessarily adequately fund them.

But, in trying to sort through, it has clearly been an emerging problem, and I am trying to figure out why there has

not been a focus or it seems to--but I have some problems similar in Indiana. We just did a meth hearing on Friday, but we also just had a major arrest of somebody who--the biggest series of bank robberies in the tri-State area I cannot remember if it was 20 banks or 30 banks. Some violent bank robbers were stealing money to buy OxyContin.

Also, some of that was not just banks--a few were banks, most of them were pharmacies. And they were very violent robberies of pharmacies related to OxyContin, which is another side thing that is happening if we cannot get doctors to prescribe it. But we need to look at this, because clearly this has been a big focus. And we have to have focus which I do not believe is the case in the law enforcement side, but let us just say there is not. I am going to say this as a Member of Congress who is perceived correctly as being friendly to the pharmaceutical industry, who is friendly to the medical industry, who believes that malpractice insurance is already driving doctors out of business and unwilling to cover certain people, and we have to figure out how to deal with medical malpractice.

But, there is a general perception in the public that to some degree the pharmaceutical companies are keeping us from correctly and aggressively addressing the subject when it comes with a legal drug. And when we are hearing in places like Florida, where this is exploded, that we do not even have a request on the table for a task force. It is a little troubling. Because somewhere in the country--you said you had a national task force, but I do not understand it. Some Members--there is a rumbling in Congress about the concerns about this too. And some internal arguing among Members.

Mr. Raffanello. I believe that in 2004, we do have a significant plus something diversion investigators, and what we have learned and what we try and do in the field is to try and use State and local partners as force multipliers, and we have been fairly successful. And that is the reality of it, we do not have nor could we ask you for the amount of agents that it is truly going to take. So we have formed alliances with our police partners and with our State people, the FDLE here.

The chief in Lake Mary sits on the narcotics and dangerous drugs of the International Chiefs of Police. We have been working with them to roll these things out. But it takes manpower, and it takes a little bit of money, and it takes time.

Mr. Souder. Congressman Mica had the subcommittee in here, he mentioned and I mentioned back when we believe it was now Speaker Hastert, chaired this subcommittee, because there has been a string of heroin overdoses in the school systems in this area, like there was in Plano, TX. And at that time there was not much focus on heroin. So part of our goal through this is to help us focus on this, but it is kind of frustrating. I want two other quick things.

One, to followup on Mr. Keller's question on advertising, and marketing, which many of us who are free market are very concerned about having restrictions placed on companies and their abilities to market. And it is--I am greatly relieved to hear about public advertising. But I am unclear a little bit on even marketing to doctors and pharmacies. Should there be and are there different standards in Schedule II, or is there any kind of mechanism internally in FDA that would have DEA and law enforcement agencies saying this drug is being abused at X

level? And what we heard today was no drug has been abused at this level, and this is a primary problem. So do we have any kind of trigger or should we have a trigger internally that says when that happens that there is now a further restriction on internal promotion and how that promotion is done? Because the inherent conflict in the free market is that somebody wants to increase their sales, not decrease their sales.

Now if there is medical malpractice problems and it is going to push up doctors' liability cost if they prescribe this drug, and then other patients are paying for it all over the place. So, you could even have a contradiction where you have a company pushing something that is driving up everybody's total health cost, because somebody is promoting something that has a higher level of risk. Do we have any current systems that restrict or put hard warnings on that are mandatory on the company? You mentioned a little black box on the thing, but frankly, a little black box on the bottle is not going to deter an addict.

Dr. Meyer. Right. Let me answer that, and I think it is a several part answer. First of all, there is no difference in the FD&C Act between how we regulate the promotion of Schedule IIs versus any other drug, so I think that was part of your question there, there is currently no difference.

Mr. Souder. And even after abuse if there is additional warnings, then there is no legal thing we would all be.

Dr. Meyer. There is no legal; right. We do internally of course, especially with knowing what we know about these, but even with other Schedule II drugs, we do pay closer attention to those in our survey of the marketing practices, than we would for drugs with less potential harm if they are misused, for instance.

With regard to the black box warning it is absolutely essential, and I made the point during my testimony, that labeling informs the marketing, and one of the things that is necessary in marketing a drug with a box warning is that box warning be prominently displayed in any marketing of it. So it is not just on the bottle, it is not just in the package insert that the pharmacist throws away, but it is actually a part of labeling. And in fact one of the enforcement actions we took against the manufacturer back in I believe in 2001, they ran a JAMA ad in the Journal of American Medical Association where we felt they had not properly displayed those warnings and we took action against them and they had to do a corrective advertisement to rectify that situation.

Mr. Souder. We clearly have a new problem in society and that is our labeling which is correct in trying to run on TV ads and other things. Now you see these TV ads that basically say this drug will make you smile more, by the way you can get liver cancer or heart disease, die of lung cancer, this and this, but you will smile more. And people are becoming immune to the labeling, let alone hard addicts, and we are going to have to deal with something beyond the labeling because we are kind of now not able to distinguish the levels of risk and the intensity of risk. And it is a new challenge for Congress.

Mr. Mica, did you have any additional questions?

Mr. Mica. Yes, just a couple of quick questions.

Mr. Souder and I participated in the development of a billion dollar drug education program that is now in effect, we have had some problems with it and we still are trying to work that out.

Dr. Meyer, you testified that education is important in this process. I am wondering, Chairman Souder, if we have a disconnect between this program that we helped create and what is happening on the streets and in our communities. Do you report in any way or recommend to the Office of National Drug Control Policy any--do you provide any recommendations in the education program based on what you are seeing happening and problems out there, because you said education is an important part--do you have any working relationship with that program, or the director?

Dr. Meyer. I would have to check to answer that, I personally do not know the answer to that. I would be happy to get you an answer.

Mr. Mica. And then the other thing would be from law enforcement. Now, you are only within the State and Miami, but DEA also, do you know any mechanism they have with ONDCP on getting information on what is currently happening to our education program, and those that are developing the educational message that we are paying a lot of taxpayer bucks to get out?

Mr. Raffanello. We do have an executive DEA agent who sits on Director Walters' staff, at ONDCP.

Mr. Mica. And you feel you are getting adequate information, but it does not sound like you are staying up with the information if you are from south Florida and we have 12 doctors on our Medicaid program that are milking the hell out of a Federal system, actually participating in the abuse problem, that gives me great concern. I have sat on this subcommittee longer than anybody. I think when Ed Towns was one of the predecessors--we have changed the title slightly--people went bananas when we had overbilling of patient's taxi service in south Florida, they were milking the billing of the taxi service for Medicaid patients. And here we actually have the program being used to produce and divert, what is it, Schedule II narcotics and our three panelists and it is sort of que paso; nobody knows what is going on.

I am going, when we get to McDonough, our State drug czar, head of ONDCP, we will have some more questions, but we need to get a little bit better coordination between the agencies and also focus on sort of the bad apples in this process. And I look forward to the recommendation I have asked for.

Mr. Raffanello. I am very happy to report I was just told by one of my people here that we are a part of the 12 doctor investigation. That fact that we were not mentioned in the paper really does not surprise me.

Mr. Mica. Well, what surprises me is that you do not know and we do need a better connect. Again, if we can go after people who are overbilling for patient taxi service, we sure as heck can go after them if they are diverting illegal narcotics that are killing our young men and women in the State and across the country.

I yield back, Mr. Chairman.

Mr. Souder. Mr. Norwood, do you have anything more?

Dr. Norwood. Just very briefly, Mr. Chairman. And I would recommend to you that you see the JAMA ad that Dr. Meyer is referring to. My personal opinion was they were--the FDA was stretching it just a little bit, but I think it would be valuable to you to see, so you can see exactly what they were considering a major mistake.

I have just one statement and I would like to know if you

guys agree with it. Heroin is illegal in Florida, but heroin is your No. 1 problem. If we were to make the manufacture of OxyContin illegal, it would still be a problem, it would only be a problem at the borders more so than in the pharmacies. It would be a problem still on the Internet.

If we were some way able to stop OxyContin from ever coming into this country, then we would again be back to dealing with Dilaudid, Percocet, Percodan and things like that. And I want to first see if you agree with that statement. Do you believe what I just said would be correct? Yes, sir.

Mr. Raffanello. If we outvote it, I believe it would come from outside the country or through the Internet from other countries, absolutely, someone would fill the void with all of the above. If you could not get it internally, than you see other drugs you could get, abused to a higher level to make up the difference.

Dr. Norwood. As it use to be prior to OxyContin.

Mr. Raffanello. This is not a new phenomenon, people have been abusing prescription drugs since we instituted prescription drugs. It is just that now there is a lot more information out there on it.

Dr. Norwood. And my concern is that we be very, very careful and not take away this, particularly I guess for cancer patients in the country. And if you outlaw it totally then the patients who actually need it and are using it correctly no longer have it available; only those who are abusing it will have it available. So all I am saying, Mr. Chairman, is we have to be very careful how we handle this problem.

And I yield back.

Mr. Souder. I appreciate that, and as we tackle a couple of things, it is just like what we had on our meth hearing on Friday, and some of our meth hearings are emerging drugs. In Indiana, for example, meth has doubled each of the last 4 years in a row. And there are ones that are growing, there are some that are relatively stable. I think it is fairly safe to say we do not have control of the south border yet, and the Carribean or the south border.

But as we think more progress, particularly on things coming through airports and through UPS, FEDEX searches and we get better control of our borders, which if we are going to have homeland security we have to do. Than we have to watch for things that we are doing internally as well, that they do not become a replacement. So if in fact we are successful in pushing Afghanistan and Colombia on the heroin question, that we do not have methamphetamine and then OxyContin replace those drugs of choice. And think ahead 3 to 5 years or 10 years. We also ought to at least have the social stigma on something that is dangerous and make sure, because part of what happened, like what is happening on so-called medical marijuana which is a substance inside marijuana that if you get something that is an illegal drug labeled as a good drug it becomes much harder. And what we have to do is separate it in the case of some of these things, that they are controlled, that only under managed use can you get them.

And what we are debating here is something that was widely spread that is now becoming more tightly managed and how, as a society, do we rein it, when at the very beginning we did not understand the nature of the risk, as I understood. That still has a huge benefit in this case and in high risk case, and we are going to face this and more. But if we are successful in

border control, we have to watch about the replacement.

I thank each of you for your testimony. We will have some additional written questions. If you want to submit anything else for the record, feel free to do so.

If the second panel could now come forward. The second is the Honorable James R. McDonough, director of the Florida Office of Drug Control; Dr. Stacy Berckes, Board Member, Lake Sumter Medical Society; Mr. Jack E. Henningfield, Ph.D., Pinney Associates, on behalf of Purdue Pharma; Ms. Theresa Tolle, president of Florida Pharmacy Association. Mr. Mica.

Mr. Mica. Mr. Chairman, while the next panel of witnesses are being seated, unfortunately the Honorable Burt Saunders, the State Senator, District 37, and chairman of the Florida Senate Committee on Health, Aging and Long Term Care, because of another emergency situation is not able to be with us today. He has notified the subcommittee. So I ask unanimous consent that his entire statement be made part of the record.

Mr. Souder. Without objection, so ordered.

[The prepared statement of Mr. Saunders follows:]

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[GRAPHIC] [TIFF OMITTED] 95555.057

Mr. Souder. If each of the witnesses will raise their right hands.

[Witnesses sworn.]

Mr. Souder. Thank you, let the record show that each of the panelists replied in the affirmative.

Thank you for coming today, we really appreciate you helping us clarify this issue. We are going to start with Mr. McDonough, I keep wanting to say the regional drug czar, so I thank you for coming today.

STATEMENTS OF JAMES R. MCDONOUGH, DIRECTOR, FLORIDA OFFICE OF  
DRUG CONTROL; DR. STACY BERCKES, M.D., BOARD MEMBER, LAKE  
SUMTER MEDICAL SOCIETY; JACK E. HENNINGFIELD, PH.D., PINNEY  
ASSOCIATES, ON BEHALF OF PURDUE PHARMA; AND THERESA TOLLE,  
R.PH., PRESIDENT, FLORIDA PHARMACY ASSOCIATION

Mr. McDonough. Mr. Chairman, thank you very much for having me and for holding this hearing. On behalf of Jeb Bush, the Governor of the State of Florida, he extends his greetings and his appreciation for what you are doing.

And to Mr. Mica, sir, thank you very much for your



suggestion that the hearing be held, it is always an honor to appear before you.

And sir, welcome from Georgia, very good to have you down here. I live only about 12 miles from your State and I love it because I can go up there and get my gas at about 20 cents a gallon cheaper.

I have submitted a statement for the record, I would like to sum up that statement, in just a very few minutes if I might, Mr. Chairman.

I think there has been adequate discussion of the scope of the problem. I would just add a couple of things that we have noted. In addition to the theft of prescriptions through the thefts of the pharmaceuticals themselves in resale, in addition to the Internet sales which we think is a major problem and to the doctor shopping, what I call pharmacy hopping, and finally in addition to the corruption we have a small amount but some in the system itself. We also have uncovered a great deal of recipient fraud in the State of Florida, and diversion at the far end, such as in nursing homes for those for whom the drugs are intended. They do not get them, and are often unaware of that and unable to report it.

Florida does have a large problem with this, I do have oversight on the extent of the problem and the problem I am talking about specifically is prescription drugs, the abuse of them and that is all of them. Much has been said this morning on OxyContin. As we are able to track this it is oxycodone the chemical compound in OxyContin and other drugs that we really keep track on, but when we combine them with the hydrocodone and the methadone, we come up with an aggregate that led to a greater death rate than heroin and cocaine.

So from my perspective, prescription drug abuse has become the greatest killer in the drug world in this State, and that is an enormous amount. There are, as you know, and you will hear later from the mothers and fathers of some of those who have died in this room. I hear from them and count the total loss as 10 a day. If we look only at the abuse of prescription drugs, devoid of any other illegal drug abuse, it is five killed per day. Unacceptable, an epidemic of first proportion. I might add what is really unnerving about this in addition to that grotesque death rate is the rate at which it is rising. So we only began tracking them in Florida in 2001, and every year we saw it go up 25 to 30 percent. I do believe we have the rise in the death rate stopped this year but it is still far too high. We are on track in 2003, to come in slightly above the numbers that we had in 2002.

So the scope of the problem is vast, it is steep, and very complex. Governor Bush had directed a series of very aggressive actions that will address it. I would just like lay out the breadth of that briefly.

First of all, we would appreciate, all the help we can get from our friends at Federal level, and I know all the people that testified before, admire them all, but I think we have to work harder on this particular problem.

ONDCP and the National Institute on Drug Abuse points out that the second most abused drugs in the United States now after marijuana are prescription drugs. That is an enormous event, it tells me it is the new wave of drug abuse. In the history of drug abuse in the United States, there is always a new way: it is cocaine, it is crack cocaine, it is methamphetamine. Today it is prescription drug abuse, and by

the way, methamphetamines have not gone away so we still have a problem with that. But it is a serious problem.

We are looking at law enforcement as a way to get at this problem, and although it did not come out clearly from our Federal friends, who are helping us, I will tell you the State of Florida is getting very aggressive in going after any corruption in the system. So, all of the doctors and I do not know the names of the ones specifically referred to in that article, but I do know that we are looking at where we believe there is an element of corruption and we are going after that. Not just for doctors, but for the pharmacies as well.

We also have, as I said, a major recipient fraud problem, which is not a light problem. A recent statewide grand jury investigation indicated that it could be a significant percent of the Medicaid system in the State and the Medicaid system in the State is something like \$13.5 billion. But law enforcement I have to point out is not enough by itself, it comes in after the fact, after people have died. So we are looking at early warning systems that will allow us to detect early through Medicaid and other data mining sources that we have a problem.

And we are also looking at process, the process that allows the administrator that oversees the system, whether it be the distribution of pharmaceuticals, the use of Medicaid, passes that off to the appropriate investigative authority when we believe we could have an instance of fraud and abuse and diversion. It is also the education of doctors. We find that many doctors do not have adequate identification capability of addiction, as well as the pharmaceuticals themselves. So, we are looking at requiring a greater effort to educate our doctors. And certainly we need to inform the public of the risk of prescription drug abuse.

So, it is the entire process that we will get at early warning, law enforcement, training, and education, and finally a legislative packet within the State that will allow us to deter the event for the most part before it happens. I will tell you that the prescription drug validation system we are looking at all by itself will go a long way to stopping the grotesque death rate we are going under. It will not completely stop it, but it is the single most important thing we can do. It is that package of events in combination with what the Federal authorities can do that I think would help us bring this problem under control.

Thank you, sir.

Mr. Souder. Thank you, very much. Doctor, is that Berckes, next.

[The prepared statement of Mr. McDonough follows:]

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Dr. Berckes. Mr. Chairman, I would just like to clarify my credentials, in addition to on the witness I am identified as a member of the Board of Governors of the Medical Society, and indeed it is via that mechanism that I was invited, but, I think it is important before I give my testimony that it is understood that I am also a Board certified anesthesiologist and pain medicine practitioner. My practice is in Florida Pain Management Center, and additionally that I am the chief of staff at Florida Hosptial, Waterman.

Since there was not an opportunity, and if you thought it was beneficial for the record I will certainly attach my CV to the written testimony if you thought that was useful.

Mr. Souder. It is always helpful to have any extra information about the witnesses.

Dr. Berckes. Thank you for allowing me to clarify that. ``First do no harm.'' Those words, from the Hippocratic Oath, take on special meaning when discussing the topic of drug use and abuse. I speak to you today with almost 20 years of experience practicing medicine, the majority of those years treating acute and chronic pain. I agreed to testify because I feel strongly that being on the front line of an issue offers a unique perspective to those interested in directing substantive public policy.

These proceedings are being followed by many that have been touched in one way or another by this issue. To those that have lost loved ones, I extend condolences. As painful as it may be we must learn what we can from each and every failure to best serve those with needs in the future. Simply banning a drug that has demonstrated usefulness is not an option.

To the pharmaceutical companies that may have an interest in these proceedings, let me say, keep your science pure. Continue efforts to provide true continuing education so we can best serve our patients. Attempts to manipulate data and words for the sole purpose of creating demand and increasing sales will ultimately fail. Do not promote the mindset that there is a pill for every ill.

To the patients that suffer chronic pain, please know that efforts continue to increase the quality of your lives. We understand now more than ever before about the neurophysiology of pain, the pain signal, pain generators and the pain process. This understanding has resulted in many more treatment options than ever before. The use of narcotic analgesics is just one of the tools that may be useful.

In my practice lifetime, I have seen the pendulum swing from one end of the spectrum to the other with respect to the use of narcotics. In the 1980's, I had to regularly defend this practice and now I am having to recommend against it with almost the same regularity. Every patient deserves to be

evaluated and treated as an individual in a way to be determined by his or her physician. Many things cannot be cured. Pain as a symptom is handled differently from pain as the disease State, which often, at best, is managed. True pain management is a dynamic process that demands continuous communication between a patient and the doctor.

To the pharmacists who fill prescriptions, I urge you to adhere to the highest level of your profession's ethics, and do not hesitate to question prescriptions that appear irregular. The system of checks and balances only works when active 100 percent of the time.

To my colleagues, you know that you are responsible for knowing the possible consequences, benefits, risks, and complications of any prescription you write. There is no substitute for the history and physical examination. The issue of diversion of legitimate prescriptions is an area in which we are not formally trained, but one in which we always must maintain a high level of suspicion when we are prescribing drugs with known street value. The judicious use of urine or serum screening to document compliance of a treatment regimen probably needs to be increased. Additionally, understanding the differences in abuse, addiction, tolerance and dependence is required for appropriate communications with patients, caregivers, as well as other colleagues and law enforcement individuals and officials.

With respect to public policy, I can only say that there is no way to legislate judgment. This is particularly true to the problem at hand. There are already laws that cover inappropriate obtaining, use, and possession of controlled substances. There are already laws that cover the inappropriate practice of medicine and pharmacy. There are already laws that cover what a drug company can say or do. Additional laws in these areas will probably not result in any substantive change in the status quo. Additional funding in specific areas to enforce laws already on the books may help.

The data base that has been discussed may have merit but the details about the design, construction, implementation, and ongoing costs have not been forthcoming. Anything that makes it more difficult for doctors to take care of patients is unacceptable. The availability of controlled substances via the Internet is one frontier which probably deserves additional legislation.

Finally, the unfortunate truth is that there are, always have been, and always will be people with the genetic makeup that fosters drug abuse and the black market that feeds it. Any system that man creates will be circumvented by man. So let us be cognizant of the law of unintended consequences when we try to make anything better.

Perhaps our greatest hope lies in the continued discoveries of the human genome project, that will let us understand the more complex areas of opiate receptors, and why people react in such varied ways to the same drug. Meanwhile, there is no better cure for the present situation, than a true understanding of the existing science, and an ongoing doctor/patient relationship.

Thank you.

Mr. Souder. Thank you for your testimony. Next we go to Dr. Henningfield.

[The prepared statement of Dr. Berckes follows:]

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Mr. Henningfield. Thank you for the opportunity to testify on the challenges posed by prescription drug abuse. I am a professor at the Johns Hopkins Medical School where I direct the Innovators Combatting Substance Abuse Awards Program. I am also, vice president for research and health policy at Pinney Associates, which is a science and health policy consulting firm.

We assist Purdue Pharma and other companies seeking help in identifying factors contributing to prescription drug misuse, abuse, diversion and addiction. We help develop strategies to reduce such unintended consequences while enabling appropriate medication use and access. I am representing Purdue Pharma to offer recommendations on this topic. The issue is important to me and it is to Purdue Pharma. The consequences of abuse and diversion of medications are serious for the people who abuse drugs, and the consequences are serious for the million of people living with pain. I have several observations and recommendations that I hope will help you. My written testimony provides these in much greater detail. There are no simple solutions, I think we have all said that, and I agree heartily.

Prescription drug abuse is a complex historic and evolving public health problem. The modern history of pain reliever abuse in America may be traced to the Civil War when the syringe revolutionized the treatment of pain, but also led some to develop addiction to the opioid drug morphine. It was then called "soldier's disease." Our Nation has struggled to find the right balance between medication access and control ever since. The history of substance abuse also reveals that the cycles are rarely anticipated and not readily controlled. For example, cocaine went from a small blip on our radar screen in the 1970's to our Nation's major drug of concern in the 1980's. Opioids such as heroin increased in the 1980's, in the 1990's prescription opioid abuse increased undoubtedly due in part to the perception that they were safer and less addictive than street drugs.

It is clear that drug abuse and diversion go far beyond the chemistry of the drug. My first chart shows data from the major Federal survey that measured non-medical use of opioid pain relievers by brand names. The short bar on the left side represents OxyContin. I show these data to illustrate the diversity of drugs that are abused and the complexity of the challenges facing us. As you may surmise and has been stated several times today, drug abusers have lots of choices and history tells us that when they are denied one drug they quickly turn to another.

Such surveys provide a general picture of the substance abuse landscape, but they have many shortcomings compared to the data that we rely upon to track outbreaks such as influenza, West Nile virus, and hepatitis. In fact the December GAO report on prescription drug abuse acknowledged these limitations concluding, "Current Federal surveys do not provide reliable, complete or timely information that could be used to identify abuse and diversion of a specific drug." Accurately estimating the numbers of deaths, and correctly attributing their cause is also critical to developing efforts to prevent future such deaths.

I would like to show a second chart from the 2003 Florida

Medical Examiners Interim Report of drugs identified in deceased persons. Some of these data have been discussed today. This chart shows the frequency of association of various drugs with deceased persons. Alcohol was associated with the greatest number at 31.7 percent, then benzodiazepines at 16.1 percent and cocaine at 14.6 percent. All oxycodone medications combined were associated with 5.6 percent. While this chart implies straight forward relationships between drugs and deaths, the reality is not so clear, as evidenced if you look at the report in detail. Determining the actual cause of death for any of these drugs is complicated and in many cases multiple drugs were evident.

Another study found that 97 percent of drug abuse deaths contributed to oxycodone drugs actually involved several drugs. In discussing these statistics I must state that any death from drug abuse is tragic, but as we seek solutions we must understand the problems well enough to develop solutions that will actually work to prevent such tragedies in the future.

Another complication in understanding drug abuse trends is that abuse of single drugs by individuals is rare. For example the overwhelming majority of persons who used OxyContin non-medically in a Federal survey had abused at least two other analgesics and/or illicit drugs of abuse, such as heroin, cocaine, and marijuana.

Let me wrap up by mentioning six key recommendations that I believe could contribute to a comprehensive solution: First, address deficiencies in our drug abuse monitoring system that were describe in the GAO report. We need accurate and timely information. Second, provide education at all levels of society about the dangers of prescription drug abuse. Third, nurture community partnerships as advocated by President Bush in his State of the Union Address. Fourth, strengthen our drug abuse treatment system so that people who develop addictions can get treatment that matches their needs when they need it. Fifth, encourage the development of comprehensive risk management programs for controlled medicines as recommended in the GAO report as well as by FDA and DEA. Finally, we need to address gaps in the drug control effort opened by unregulated Internet sales.

So, in conclusion, let me emphasize that prescription drug abuse and diversion is an important public health problem that warrants increased attention. There are no simple answers. As we move forward in search of strategies to deter abuse and reduce diversion we need to recognize the needs of people in pain as well as the health care professionals who treat them. I believe that these actions need to be part of a comprehensive solution to the problems of prescription drug abuse.

Thank you for the opportunity to testify.

Mr. Souder. Thank you for your testimony, and we will make sure your entire written testimony appears in the record, and if you have additional materials too.

Ms. Theresa Tolle, is it Tolle.

Ms. Tolle. Tolle, it is Tolle, yes.

Mr. Souder. President of the Florida Pharmacy Association.

[The prepared statement of Dr. Henningfield follows:]

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Ms. Tolle. Thank you, for the opportunity to be here today. I am Theresa Wells Tolle, I am a pharmacist and I am co-owner of Bay Street Pharmacy, which is an independent pharmacy in Sebastian, FL. I am the president of the Florida Pharmacy Association, and today I am here representing the American Pharmacists Association. APhA represents more than 50,000 practicing pharmacists, pharmaceutical scientists, student and pharmacy technicians. And we are the largest national association of pharmacists in the United States.

APhA welcomes the opportunity to present the pharmacist's perspective on the abuse of prescription drugs, including controlled substances. As the medication experts on the health care terms, and the health professionals dedicated to partnering with patients to improve medication use, we appreciate the opportunity to discuss the importance of striking a balance between providing effective, legitimate, appropriate health care and preventing prescription drug abuse and diversion.

Prescription medications are safe and effective when they are used appropriately, and pharmacists are the health care providers who work most closely with patients to make certain patients use their medications appropriately. Prescription drug abuse is one type of medication misuse, misuse that we as pharmacists try to prevent. Pharmacists work collaboratively with prescribers to prevent the diversion of prescription



medications and to identify incidents of abuse or addiction. As part of this process, pharmacists assess the appropriateness of every prescription order they review or dispense. I watch for individuals who attempt to fill fraudulent prescriptions, who are visiting multiple prescribers, or present prescriptions for unusually large quantities of medication. Every day, I assess the validity of prescriptions, by watching for errors in the content or the format of the communications. However, it is not always easy to determine if a prescription is legitimate, and I cannot view every patient as a potential drug abuser without compromising my responsibilities as a health care provider.

Identifying potential drug abusers is an area where collaborations with regulatory agencies makes sense. For example, the Florida Department of Health recently barred one of Florida's most prolific Medicaid prescribers from issuing any more prescriptions for controlled substances. Having either the Florida Board of Medicine or the Department of Health provide this information to the pharmacist community would help educate pharmacists about potentially illegitimate prescriptions.

Another area of collaboration between regulatory authorities and pharmacists is now occurring in my own practice. The narcotics detective of our local Sheriff's Department informs pharmacists about potential drug abusers as well as when a local prescriber's prescription blanks have been stolen. They do this with a fax alert. These efforts help pharmacists determine whether a prescription is legitimate. In both of these examples, the regulatory authorities are helping pharmacists by providing them information. However, in both examples the pharmacist has the final say in whether or not the prescription is for legitimate purposes, a determination they must make for every prescription presented to them.

APhA supports efforts to strike the balance of reducing prescription drug abuse and diversion, but without restricting patient access to drugs. In October 2001, APhA, in collaboration with 20 other health care organizations and the DEA, released a joint consensus statement on the need to prevent abuse of prescription medications, while ensuring that they remain available for patients in need.

Focusing on the subset of medications known as opiate analgesics, the groups recognized that for many patients, opiate analgesics are the only treatment option to provide effective and significant pain relief. However, a narrow focus on the abuse potential of a drug could erroneously lead to the conclusion that these medications should be avoided when medically needed, generating a sense of fear rather than respect for their legitimate purpose.

We caution against efforts to restrict the distribution of certain medications or arbitrarily limit health care providers' ability to prescribe or dispense appropriate medications. With every barrier erected to limit diversion, the potential for those barriers to diminish appropriate prescribing increases exponentially. Reduction in the drug distribution process can delay access to medication therapy, and disrupt existing patient-pharmacist-prescriber relationships. Additionally any stigma attached to the drugs will have a significant chilling effect on health care providers' willingness to prescribe and dispense appropriate medication and patients' interest in the medication.

In a survey conducted by New York State's Public Health

Council, 71 percent of physicians surveyed reported that they do not prescribe the most effective pain medication for cancer patients, if the prescriptions require a special State monitored prescription form for controlled substances, even when the medication is legal and medically indicated for a patient.

Efforts to limit abuse and diversion should be developed in collaboration with health professionals and consumers and designed for maximum benefit and minimum intrusion. State level tracking systems when well constructed can provide this benefit, and well constructed programs provide prescribers and pharmacists with relevant timely information about dispensed medication. We cautiously support efforts to heighten regulations in this area. Federal enforcement agencies such as DEA should continue to be a law enforcement agency fighting the illegal diversion of drugs. But the DEA should not be turned into a medical oversight body. Drug therapy should be managed by health care professionals.

The very threat of regulatory intervention and oversight and the fear of having their intentions misconstrued could dissuade physicians from using aggressive efforts that are often needed to use medications effectively.

It is important that patients do not lose access to medications because of a failure to prevent medication misuse. Solutions must not have a chilling impact on the effective drug therapy management. The solution requires the education of health care professionals, law enforcement personnel, and the public on the use and abuse of prescription medication.

APhA, and its members are committed to working with Congress, the FDA, the DEA, and other health care providers and patients to find the appropriate balance between appropriate medication use and measures to curb the abuse and diversion of prescription drugs.

Thank you, for your consideration of the views of the Nation's pharmacists, APhA, looks forward to working with the committee to develop a safer and more effective system of providing prescription medications to all Americans.

[The prepared statement of Ms. Tolle follows:]

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Mr. Souder. Well, I thank you each for your testimony. And I want to say up front, which you heard me say in my first panel, I really did not come into this hearing with much of a preconceived notion. I have seen some of the headlines in my own district. We focused on a lot of other issues and so I was not as knowledgeable as Mr. Mica or Mr. Norwood in the particulars. And among other things Eli Lilly, is a major player in Indiana and I have been a strong supporter of Eli Lilly. In the interest of full disclosure I have anesthesiologists and all sorts of different doctors on my

finance committee, because when we ran for office in 1995 there was a lot of outrage about the nationalization of health insurance and so I am disproportionately hooked up with them.

Medical Protective, one of the largest malpractice insurers for doctors is based in my district, along with General Electric. But I am frustrated by your testimony. I have been getting the crap kicked out of me, with all do respect, for working too much with the medical industry. If the medical industry cannot understand the difference of a drug epidemic and wants to stand behind the shield of do not intervene, we are going to do some nice compromises in a drug epidemic, you bring a lot of the pressures on yourself.

I do not like a lot of what we in Congress passed in HIPAA regulations. I am tired of all the paperwork on every little thing, why can we not prioritize. OxyContin, right now is a priority type of thing, or the underlying thing underneath it. It is not aimed at Purdue Pharma, it is not aimed because it can spread. But let us lay a couple of things out in the record here. The difference between a heroin dealer and a cocaine dealer, is you are not them. You are dealing with prescription drugs that are paid for mostly by other people. One difference is that is the Federal taxpayer as an individual taxpayer do not pay for cocaine and heroin. There is a ethical difference when you ask the Federal Government, the State government, and other taxpayers to subsidize somebody's habit. That is going to bring additional pressures on that.

Second, that when the network is a legal distribution network that is approved by society, that it is going to bring different pressures on it. Now, it is absolutely true that the anesthesiologists, and people who understand pain reduction have to be primary players at the table, and that pharmacists cannot assume that everybody coming in there is a criminal. I appreciate that statement. On the other hand, when you have an epidemic in the community and when small town pharmacists are being held up at gunpoint in my district, and that in fact a fair number, which has not been established what percent, are in fact criminals who are doing it. It suggests that you are going to have to use a little more discretion. There is going to be some regulations with it, or what is going to happen is the entire pharmaceutical industry, the entire flexibility of the medical community is going to be taken away because the general public is not going to tolerate their money being spent. Which is different than cocaine and heroin. I am not arguing here that it would not move to something else, but we have the obligation as stewards of the taxpayers' dollars, to at least make sure our dollars are not being used this way. To make sure that those who are in legal trade are not. I am particularly outraged at the statement that 71 percent of the doctors in New York State would say that they would not prescribe what is best for their patient based upon on a paperwork decision. And quite frankly, that leads us into a question of should their malpractice insurance go up.

In other words, maybe one of the ways to do this is to have a different criteria on the people who do not prescribe because they do not want legitimate paperwork. And to me, part of our responsibility in oversight is we have dumped so much illegitimate paperwork, chasing at the margins on the doctors, and quite frankly, by not controlling the lawsuits all the time. Because you can be harassed for everything, and that is part of the concern here. That sometime this could lead to a

bunch of lawsuits on the drug company, on the pharmacist, or the doctors which is outrageous. The problem is we need to take responsibility how to address this, get control of the lawsuit question, but that should not prohibit us from trying to address legitimate concerns in that.

We can make some progress as we talked about here which you all have supported, but the underneath is what has caused me to erupt here as a friend, and say, look, this is different. What we are looking at is an epidemic, and if we do not try to treat epidemics like this, that our whole support system for not cracking down and having national socialized medicine system is going to collapse.

Because if we do not go after the bad guys aggressively and target those higher risk groups first and foremost. And we do not have a mechanism to identify those high risks. In other words, if you will not help us go after the highest risk areas, then everybody will become a suspect. And then there will be non health regulation by DEA in the pain relieving medicine. This cannot be kind of like a slap on the wrist and we are going to put a little warning up here and so on, because it is not going to work. The outrage of the community already over the general cost of prescription drugs, the general cost of health care is so large that we are walking on a very tight wire now. And having this kind of thing on the top of the other pressures on health care is going to bring consequences far beyond whether we tinkering around with OxyContin.

When we have an epidemic erupt we need everybody working together and saying we are going to focus on this right now. I would like to hear some of your reactions to that. Who wants to start.

Dr. Berckes. I cannot speak to the 71 percent that was mentioned earlier and I hear and I understand from your perspective as well. The majority of the physicians that I have spoken to with respect to this issue when it became clear that I was going to be the one to talk about this today is that doctors do not want to have tools taken away that can help. And indeed I can tell you that the percentage of physicians that are responsible is very small.

I think we heard testimony about that, things have been published already. But I can also tell you the frustration from the point of view of a physician that cooperates with the Florida Board of Medicine in looking at these outrageous cases and what has to be done and the hoops that have to be jumped through to pull their license. OK, looking at records and I have cooperated with the board and I am glad to do that. There is not a lot of pain management physicians that have the qualifications and that are volunteering with that, and it takes a lot of time.

I have seen things and it is just beyond me how a doctor can get away with it for so long. The only thing is and I think Representative Norwood, brought this up earlier, it must be that the pain that they are going to incur is very small to the possible benefit. They are not going to hurt enough and it is not just taking away their license to practice medicine, but it is throwing them in jail, and I do not think it has been done enough, and I have seen enough and it makes me sick. But we apparently have a process that protects those that are causing the problems. And much more, it is much more of a problem than probably we suspected before.

When somebody walks into a pharmacy and presents a

prescription for 540 OxyContin 80 milligrams month after month after month, there is something wrong with the whole process: where that originated, who is filling it, the whole thing. I mean it is just mind boggling. We do not want these tools taken away, and we know that they will be, and we are sensitive to the health care dollar. The health care dollar be it Medicaid, the future Medicare prescription benefit, we do not want this taken away, and we support any efforts that may enable the situation to get better.

Organized medicine supports this, please do not misinterpret anything I have said. We just do not want to go back to having our hands tied behind our backs, OK. The evolution of the speciality of pain medicine has been a relatively new one. And I believe patients are being served better, whether it is cancer patients or other non malignant type of pain, non-cancer pain. And at least what I see in my community is that there is less use of certain of these drugs by primary practitioners, and they are allowing the people with specialized knowledge to make the calls on this. And whether that is something that is a statewide trend or a nationwide trend, we have been led to believe because of the proliferation of pain management specialists that is happening. But when you see things like these incredible numbers of OxyContin being prescribed by small numbers of doctors one has to believe.

So, the one area that I am frustrated with as I have tried to think has to do with this Internet thing. I mean every time I turn on my computer and answer my e-mail I am offered all kind of things. I mean I do not know how that is regulated but that is a problem that I do not understand. Having a data base, a computer data base, is something I think certainly can help. But who pays for this. The money in the pie for health care is already smaller or at least relatively smaller because there are more people that we have to take care of with the same amount of money. So, who is going to pay for that system.

And I have seen things--you know, if we include oxycodone, just Schedule IIs but we do not include Schedule IIIs, we have shot ourselves in the foot. I mean, I can tell you, using hydrocodone is just as risky as using oxycodone.

I mean there is--and for the people who abuse it is the same thing. Why one is a II and one is a III, I am sure there is some interesting history about that, but if it is comprehensive I think we all can probably get a handle on it. But we have these issues of HIPAA that we are all dealing with HIPAA right now, and I do not know which way I am going with HIPAA. I know I am afraid of violating laws with HIPAA, and I do not know how that would equate. But we should be able to with the resources of the Federal Government, the United States, be able to coordinate with those areas that are mandated by each State, to get a handle on this thing really quickly.

I firmly believe that and I pledge my support.

Mr. Souder. Probably, having to list our payd when we go to the doctor is a over-regulation of HIPAA. That is the way it seems sometimes.

Ms. Tolle. Dr. Berckes, did a great job in covering on a wide topic in a short period of time. I think definitely--one of you mentioned earlier an umbrella organization with the Federal Government, and then State control of that umbrella organization. To me that makes the most sense. Colonel McDonough said that there is controlled substance monitoring

legislation that is proposed in Florida and I know there is in other States, I believe 18, there may be more, that currently have that kind of system in place.

I think if you can get something like that in place where at least you have an ability to look and see who is doing this, who is prescribing, the patients who are abusing the system. Yes, I have concerns with HIPAA and privacy violations, but I also, think at least there is an ability for us to know. There is a way for us to, a place for us to go to. We have groups like the Florida Department of Law Enforcement, who could be the coordinating group for that in the State of Florida.

As I mentioned, we are very fortunate in my county, because I have a Sheriff's Department that is very proactive, and they work with us and that works very effectively. I had a doctor who was closed down Monday a week ago, their controlled substance ability--or his ability to write controlled substance prescriptions was taken away from him. I knew that within 2 hours of that happening, because my local law enforcement agency let us know that. At the same time a pharmacy was robbed in our area, and we knew that as well, we also knew that the pharmacist recognized the suspect and that person was being questioned. Which kind of helped us breathe a sigh of relief that perhaps he was not coming to us next. But I think those types of coordinating efforts are very helpful. And I see that as an opportunity for us to move forward and solve this problem. I can tell you that there are people out there who are writing those 540 tablets of OxyContin, and unfortunately there are pharmacists who are filling them month after month, and there should be penalties. We need to make sure that those people are afraid, that they are going to be penalized.

Mr. Souder. This is also happening in meth, where we had one case where one of the biker gangs that have been developing a network of meth labs went to pharmacy training and got control of a pharmacy. And we have to be able to weed out the at risk groups so that we can keep the harassments down on legitimate pharmacies. To do that there has to be cooperation and information. Dr. Henningfield.

Dr. Henningfield. Congressman, I agree with everything you said. I think that we do have a serious problem with prescription drug abuse, and we do need to address it.

I have a couple of suggestions, I would like to keep an image in mind, and the image is a balloon. And what we have to be careful is that we do not squeeze the balloon in one place so it pops up in another place, because that is what happened over decades with drug addiction.

We have some serious problems in our infrastructure, our monitoring system. We would not tolerate a CDC that told us a year or two after the fact when there was a new virus or epidemic, or hepatitis outbreak. We expect comprehensive rapid, reliable monitoring for drug abuse. We have that for other diseases. We have made a lot of progress, I think the institutes have made a lot of progress, but if Congress further prioritizes this I believe that SAMHSA and other Federal agencies could do a better job and do a better job of integrating local information with Federal information as the CDC does.

Monitoring deaths and correctly attributing them is critical. The Florida Medical Examiners report, if you look at it in detail, you see that ascertaining actual cause of death is a complex business. Yet, as CDC knows with other diseases,

you have to do that if you are going to fix the problem and prevent it in the future. We need a better, more systematic way of doing that.

The Internet is a hemorrhage, I do not know how to fix it. Prescription drug monitoring is a national system and a local system, that allows doctors to find out, how does this integrate them with our Federal monitoring systems. On treatment, our former surgeon general Dr. C. Everett Koop, he said, ``it is easy to get addictive drugs, it is hard to get treatment; as a Nation, our challenge is to reverse this.'' That is a fact right now, and that means when people do get into trouble and they will get into trouble; no matter what we do, there will be some people in trouble. They have to have a place to go when they need it, and it has to be the right kind of treatment, and the one thing that has not been discussed directly today is also a conclusion of the GAO report and FDA, and DEA, and that is the concept of risk management programming.

The whole idea is the Controlled Substance Abuse Act came about when a lot of these problems were not on the radar screen. It took a simplistic approach, it is basically the chemistry. My laboratory at NIDA studied mainly the chemistry, and addiction potential. Now, we know it is much more than the chemistry. The concept of risk management programing and plans is that you a, identify all the potential risk associated with the drug; b, you develop solutions to the best of your ability to minimize those risks and still maximize the beneficial effects of the drugs.

Then you should have a monitoring system in place to fix it if it does not work. And if you do not have all that, you will have problems and they will recur and recur and recur. You could take the top 10 drugs of abuse, licit or illicit, off the market, ban them, and they would be replaced. You would just be squeezing the balloon in one place. So, I urge you to consider a comprehensive solution. There are things that you can do.

Mr. Souder. Mr. McDonough, do you want to comment on this?

Mr. McDonough. Mr. Chairman, very briefly, I could not agree with you more, the death rate is obscene. We do have to take steps and have to take strong steps immediately. We cannot hide behind the excuse that we have to be very careful as we go forward--it is an epidemic, as you said. When you are dealing with an epidemic you have to take immediate action.

I would point out the validation system in the 18 States and the one we expect to put into place in Florida, is most used not by law enforcement, but by doctors. Doctors want to know what their patients are being prescribed, only then can they give good medicine. And since we have worked very closely with the Florida Medical Association as well as with the pharmacy folk, we know for sure that neither group tolerates murderers in their group. I will point out that Florida has been very aggressive in going after this from a law enforcement perspective and in identifying the extent of the problem.

That means, therefore, we get a lot of press on this. I suspect that these problems exist throughout the country, but I know that is why you are looking at it. Here for the purpose of addressing the issue for the entire Nation, and I laud you for that.

I also wanted to point out that it is very easy to play with data, although, it was reported that most deaths are poly drug deaths, I will tell you for sure in Florida, no kidding,

that for half of the prescription drug deaths, the medical examiner identified a lethal presence of the prescription drug, the chemical compound in that. So, although there may be an attempt to lose that in the wash, forget it. It is the prescription drug in one half of those 3,200 plus deaths, that killed them. There may have been other drugs present, but it was the prescription drug that killed them.

Mr. Souder. Could I get a verification on that?

Mr. McDonough. Yes.

Mr. Souder. Would the prescription drug that killed them, if they had used that alone, or was the prescription drug on top of what they had in their system.

Mr. McDonough. Well, the doctor that does the autopsy says it, present in a lethal amount. Meaning that if oxycodone was present in the bloodstream, it was there in sufficient quantity to kill them.

Mr. Souder. Alone?

Mr. McDonough. Alone. The other drugs I guess they added that for the high. I might add it is very difficult to ascertain which was the prescribed drug that killed them. Because the autopsy does not go into the degree of investigation that a law enforcement person might. But it does appear to me that a predominant killer in the oxycodone deaths, is OxyContin. So, you are right to stress that. There was a series of articles published in the paper here in Orlando, that was able to trace a number of deaths, several hundred. And it gave a figure based on that review, an in-depth review, some 83 percent of the deaths they reviewed with oxycodone in the blood system, was traced to OxyContin. Therefore the author of that concluded it was OxyContin that killed them.

I stress this because it is so easy to talk about the caution we must exercise, of course we must exercise caution. But the fact of the matter is we are seeing 10 dead a day. So, if you are too cautious in preserving--that is one State, preserving that 10 dead a day, what you do allow to do--and not you, sir, of course--but the collective we, we allow those 10 to keep dying. Unacceptable, we have to be more aggressive than that, I do think that we can preserve what I call the three P's. No. 1, pain treatment adequately done. No. 2, the privacy of the patient, and No. 3, the sanctity of the patient and the doctor and the relationship that ensues between those two.

After 3 years of working this in Florida, I have very little patience for that raised as a new concern. That is why we had every player come to the table and every player lay out their association's, their group's concerns, I think we have addressed them all. What we have not yet adequately addressed is 10 dead a day. That is where we have to get and we have to get there in a hurry.

Thank you.

Mr. Souder. Mr. Mica.

Mr. Mica. I will just continue, Director McDonough. I was quite stunned by the first panel, it seems there is great disconnect, at the Federal level, at least from enforcement. We had one of the chief DEA officials here who did not know about the extent of the problem. And then I guess the newspaper or media has revealed some of what is going on and it does not appear it is a priority to pursue that. You are our chief officer dealing with the problem of substance abuse in the State of Florida, what specifically would you recommend to fill the gaps, now the State has their agenda and I think we will



have some testimony from a State Senator that we are going to submit to the record, as far as what the State intends to do. What specifically can we do to deal with again, the medical profession, whether it is a doctor, a pharmacist, or someone who is prescribing these legal narcotics in quantities that are killing people--what can we do from the Federal level, where do you see the gap? How do you see us filling that gap?

Mr. McDonough. I would say about three major things you could do in short order, sir. When I worked in ONDCP I was glad to take counsel and guidance from you. ONDCP has made this a priority, I think it could be stronger. It ranks up there, but from my vantage point it is the most deadly drug problem we are seeing in the country right now.

Mr. Mica. I do not know, Jim, if you were here when I talked about the disconnect, you know, you were around when we had the National Drug Education Program. It seems to me there is a disconnect there. As Dr. Meyer testified that part of this is education, and it is, but it does not appear that the Federal level we are able to shift gears to get information out. Do you see that problem and how do we address that?

Mr. McDonough. I do see the problem. I think you have the power to do that in very effective ways. First of all, is to have hearings such as this and second, to give direct guidance. I do not necessarily think it takes another law to do that, but, of course, when you stress it, when the Congress of the United States makes it a priority concern for whatever agencies respond to you at that level it becomes a concern as well.

Mr. Mica. But there is no--again, I see something missing, I loved your reports and all when you were with ONDCP, but by the time we get them the information is old and by the time we hold hearings on it, we are looking at--and the deaths figures I have are just dramatic off the charts, in the last couple of years, on this problem. So, we have not gotten the message in Washington, our Federal agencies are not responding whether it is law enforcement or others, and we do not have a program in place. So, there is something wrong there and I think we need to get with John Walters and others to see how we could do that.

The second motive in question was dealing with the bad apples who are--and these things are not coming on the market just accidentally or through the Internet. We have cases of physicians or pharmacists prescribing or issuing incredible amounts. What do we do with the bad apple, from the Federal level?

Mr. McDonough. Well, I think you need to go after any crook, and not just at the Federal level. Certainly that needs to be done, but along with State and local jurisdictions as well.

I would suggest, sir, if you work with the American Medical Association on this, they would be in the forefront of wanting to crack down on those among their ranks that would violate the laws.

Mr. Mica. Well again, I think we heard sort of the evolution of narcotics substances and the treatment of pain, and the lack of the law to keep up with the enforcement problem. That is part of it and that is going to require some adjustments to Federal statutes and laws, which I think--I do not know if we will get the cooperation of some of the medical professionals, what do you think?

Dr. Norwood. John, I think----

Mr. Mica. They are not under obligation.

Dr. Norwood. I think the people who should be and I believe are most concerned are those that prescribe medications. We are talking about 12 doctors from Florida, well that helps ruin the reputations of thousands of doctors in Florida, and they want and the pharmacists too--we want these people caught, dealt with.

Mr. Mica. Take their license.

Dr. Norwood. Well, no that is not enough. Taking a license----

Mr. Mica. Someone said in jail.

Dr. Norwood. Well, what I said is they could practice in jail. Just simply taking their license makes the problem worst, it drives them underground.

Mr. Mica. Let me just conclude with a question, and I talked to a couple of pharmacists about the problem, and some pharmacists do respond, others are concerned about liability or they have other concerns. They see prescription shopping, they see over-prescribing of medication, what can we do from the Federal level, or is this a State issue, to protect the physician--or the pharmacist, but also, allow the pharmacist who sees this activity to be protected?

Ms. Tolle. One of the things that was mentioned by one of you earlier was this--and I think it was Chairman Souder--the systems that are in place for payment of pharmacists through third party companies like where we submit an online claim and we get some information back, that the claim has been adjudicated and we are going to be paid for that. And I know that is part of your outrage, is that insurance companies and Medicaid are paying for this illicit use. One of the nice things about those programs too is that they send us alert messages back, and that really helps pharmacists. Now I do not know what the Federal Government can do, per se, but what you need to be aware of is that there are systems in place already where we are transmitting a prescription claim and getting it adjudicated, and it seems to me that a system like we are talking about with this controlled substances monitoring would--you could do something very similar you could transmit and get some sort of message back about what this patient had received or something like that.

I think that the bill that Representative Norwood has proposed to provide funding----

Dr. Norwood. It is a draft.

Ms. Tolle. OK, I am sorry.

Dr. Norwood. Work in progress.

Ms. Tolle. I have seen the language, or I have seen the draft. I think what is being considered right now is a great idea. I think you are moving in the right direction with that. By helping to fund the States that are willing to do that, and I do not know if it could be a Federal program or if it needs to be State by State. But I think encouraging States to do some sort of monitoring program to allow, to help their professionals to get that message, to know what is out there.

And of course I agree with all the efforts to do educations, I like what was said about the genome project and what we are going to have in the future to identify perhaps before it ever happens, the people who are going to be subject to that, I think education is definitely a big part of it. In the whole mental health and the issue of depression and identifying patients who might be prone to it so we can stop it

before it happens.

Mr. Mica. Do pharmacists need some protection against reporting folks, because I have heard that is also a problem, that they are reluctant sometimes.

Ms. Tolle. I guess there is always a possibility of a pharmacist being concerned about liability, but if you are reporting somebody who is obviously violating the law, I do not know why there would be a liability concern.

Mr. Mica. OK.

Ms. Tolle. I mean there may be pharmacists out there who have that concern, but it becomes pretty apparent after awhile, when a physician is prescribing outside the normal limit.

Dr. Norwood. Mr. Chairman, would you yield on that subject?

Mr. Mica. Yes, go ahead.

Dr. Norwood. Let me just point out and I have been working on this bill for awhile and our biggest single concern is liability in HIPAA. If we cannot get the job done, it is going to be for that reason.

Mr. Mica. OK, and then just--I am through Mr. Chairman, but while I have Ed McDonough, here, one of the most startling things I have learned today is that we have a Federal program, Medicaid in this case, we learned is being abused--actually a major conduit to putting lethal prescription drugs on the market and some years ago in fact our subcommittee or the predecessor of this subcommittee did a lot of work with the Florida Legislature in getting--Florida officials in getting a Medicaid task force, fraud task force. I do not know if that is still operating we had \$1 billion between Medicaid and Medicare, in over-billing and fraudulent charges. Certainly if we have people dying as a result of distribution systems being set up through a Federal program for obtaining these prescription drugs, it should be the focus of attention.

Is it still in place? And if you do not have that information now I would certainly appreciate you reviewing it.

Mr. McDonough. No, sir, we have it, and we can do a better job with it, and we resolve to do a better job with it. We have a Medicaid fraud unit. The way the system works the Agency for Health Care Administration in Florida takes a look at the data. If you recognize something should be passed off for investigation, it needs to be done in a timely fashion.

Mr. Mica. I am aware of the procedure, but are they now--this is outside of some of their original purview and purpose but certainly, you know, it is against any policy that we would promote at the Federal level to have this going on. Are they pursuing----

Mr. McDonough. They are. If Senator Saunders had been able to come today, he would have laid out a number of hearings he has held. They were very well done hearings, in which he has given great incentive for the system to coordinate better, and he will now back that with a series of laws that will further strengthen it. Part of his appeal to you was to ask for the Federal laws in the Medicaid systems that would make the penalties appreciable should someone try to do the very thing that we are talking about.

Mr. Mica. Well, thank you. And we will take his testimony and recommendations back and your suggestions. Appreciate the panelists and I yield back.

Mr. Souder. Thank you. Dr. Norwood.

Dr. Norwood. Thank you, Mr. Chairman. Mr. McDonough back to the 12 physicians again and I do not want to belabor this but I

am curious. Let us say they were indicted and found guilty or even one of them was. In Florida law what would be the penalty?

Mr. McDonough. If there were deaths involved most likely we are looking at manslaughter. In fact, we had a historic case of manslaughter, one doctor in Pensacola, four counts. I actually think there were 11 dead associated with his practice. But if there is a deceased, it is manslaughter, and then the requisite penalty that comes with that, a long time in prison. Now, it is difficult to get a manslaughter case, as you know, and even harder to get a murder case. But we are looking at that as well.

Dr. Norwood. Well, simply the overuse or allowing the overuse of Schedule IIs and IIIs where there is not a death incurred but, however, we see clearly from the record this particular person is way over-prescribing this drug, what can you do to stop it before a death occurs?

Mr. McDonough. You get into the gradations of when a crime is committed. Was it lack of education, was it an administrative problem? If it is at the lower end of the spectrum, then the Board of Pharmacy, if it is a pharmacist can move, or the Board of Medicine, if it is a doctor can move. They can suspend that license or revoke that license. Since it takes a while to revoke a license, in extreme case of administrative error, most likely the Secretary of Department Health would revoke a license. If you cross the line into criminal activity, then you can prosecute for the violation of the law. You cannot be a drug dealer under any law, a drug pusher.

Dr. Norwood. So, it is criminal activity to start with.

Mr. McDonough. At that point that I just described yes, when you were wantonly pushing the drugs knowing you do not have a legitimate patient, you have done only a cursory or no physical examination, when it is done on such a scale that the rational man would say this guy is pushing pills, you have a case.

Dr. Norwood. How many deaths in Florida, from OxyContin occurred from people taking OxyContin in a prescribed manner?

Mr. McDonough. That is a very tough question, I do not have an exact figure.

Dr. Norwood. You need to be real sure, do not guess on that. Now that is important. There are many drugs--penicillin will kill you. And it can kill you taken in a prescribed manner with antiphyllactic shock. There are many, many drugs out there that were used, thank God every day, but they can kill you in normal usage and there are that many more that can kill you if you are over-taking the particular drugs. I do not know how many--Doc, do you have any idea how many medications are available out there to health givers that actually cause the deaths of patients if taken in an overdose?

Dr. Berckes. Virtually everything that is a prescribed drug and many things that are not prescribed drugs have the potential to cause death.

Dr. Norwood. I guess water can too, you know, taken in an overdose.

Dr. Berckes. Right.

Dr. Norwood. Let me ask you--this is just a simple question I am curious about, I know you are a particular expert in pain management, I also know though physicians do not get through medical school and all the subsequent training without having a fairly good idea about some pharmacology. Maybe some in New

York, but most of them I know about have a pretty good education in that. Do you really think there is any physician in Florida that would not understand that there are dangers in some of these drugs in terms of being addictive. Do you think they are actually out there practicing medicine that do not know that?

Dr. Berckes. I think that there are a lot of--there are many physicians that do not understand the potential, I am not making excuses for them.

Dr. Norwood. I do not see how you get through med-school and not understand the potential at least for addiction they may not understand it at the level you know, but they know when they write that script for, you know, Ms. Jones, we have to be careful here.

Dr. Berckes. There is a couple of things. First of all, there are a lot of studies that have shown that when narcotics are used to control pain, you do not get the addiction. There is a small percentage of people predisposed. But I think speaking of the larger issue and I try to avoid using brand names, but OxyContin is one we can not avoid. I believe because I was in this boat when this drug was rolled out, despite the education that was provided by Purdue, those of us that are using narcotics are very familiar with a sister drug, called MS Contin. MS Contin is made by the same company, and it is morphine sulfate. Classically one of the advantages of MS Contin versus immediate released morphine is that the abuse potential was virtually eliminated, because of the sustained release preparation that this company I assume patented. There was not the ability for it to be abused, or it markedly decreased.

A lot of us believed incorrectly that using oxycodone in the form of OxyContin would afford us some of that same protection. The sustained release chemical in the way that oxycodone is released in the OxyContin it turns out is nothing like the MS Contin, so I believe there was a lot of confusion where there was intent to prevent the abuse, potential abusing oxycodone preparations by using OxyContin. We inadvertently did just the opposite.

I do not believe, I am sure there is a lot of scientific data that they had to go through with the FDA to get there. I do not believe there was any deliberate misinformation put out there, but this was an unintended thing, just to clarify.

But indeed there are doctors that think they are doing the right thing, and one of the other things especially that I have noted with this drug, when for whatever reasons you calculate the drug and you maybe are giving a little bit too much, and patients forget when they take medications. I forget, when I am prescribed by my doctor, if I do not write it down. All it takes is taking an extra OxyContin if you are already getting the higher level and you take another one you are dead in a few hours.

Dr. Norwood. I have a few more questions I have to get answers to, and a quick answer on this. Severe pain, moderate pain, the FDA refers to that a lot. I have never understood how you actually define severe pain and moderate pain. One patient has a problem that can be solved by an aspirin and the other patient has the same thing and they need a barbiturate, how do explain that, can you use severe and moderate in a sensible way? Because what is severe for one patient may be absolutely moderate for another. Do we understand that yet?

Dr. Berckes. These are subjective monitors, OK. There is no easy way.

Dr. Norwood. But that is not how FDA writes it.

Dr. Berckes. No, and I think there is too much wiggle room there and I do not know how to--we use classically and it is being incorporated as the fifth vital sign, the visual analog scale of pain. Where 10 is the worst pain imaginable and 0 is no pain. But we know that people report differently. The same pain is reported differently because of their different thresholds, because of the way they are made up. There is no way to use just one pain measurement OK, to say for sure what this is. So we use historical precedent. We know that a crush injury of an extremity is certainly different than the surgical wound caused to fix a hernia, and these are all different things. This is, sir, the art of medicine, trying to hook it together with science, and there is no way--especially in this whole area of pain medicine, there is no meter that I can have a patient put their hand on and I can tell where their pain is. If there was I think we would have a better way to handle it.

So, it is the subjective complaint and following patients on a very close basis that you are going to do the best job.

Dr. Norwood. Well you answered it how I wanted you to answer it, and I particularly wanted----

Mr. Souder. Would the gentleman yield?

Dr. Norwood. Of course.

Mr. Souder. I am fascinated with this subjective question because to me, the greater the addiction potential and the greater that we see abuse of that I would think that you would move toward a tighter application at the medical profession. For example, I just had hernia surgery, I was being asked all the way through, at least as well as I remember and afterwards as far as my pain medication, what level of pain can you tolerate. The answer is you want to tolerate no pain.

Dr. Norwood. Correct.

Mr. Souder. And so, if you are given choices you will keep taking it. The question is that if something is highly addictive and been abused, should the standard ratchet up, other than the individual identifying, which is kind of underneath. If this is an art, should the art be more constrained the more high risk you are----

Dr. Norwood. Part of the problem, Mr. Chairman, is, at least in the 1970's I think health care givers were overly constrained and a lot of people suffered during those years, because physicians and dentists alike were very hesitant to write some of these prescriptions for the very reasons that we are here about. On the other hand, there is a moral obligation as a health care giver to try to deal with the pain the best you can, and it is subjective. I just want to be careful that when we start legislation in Washington we remember that. The FDA in my view tries to make it black and white and it really is not that.

Ms. Tolle.

Ms. Tolle. Yes, sir.

Dr. Norwood. Ms. Tolle, do you have a computer in your pharmacy?

Ms. Tolle. Yes, sir.

Dr. Norwood. Do most pharamcists today in Florida, have computers?

Ms. Tolle. Yes, sir. My understanding is there is may be a few in south Florida, that are primarily Latino pharmacies,

that may not be computer based, but I would say probably 95-plus percent at least maybe greater.

Dr. Norwood. How would you operate today without a computer----

Ms. Tolle. I have no idea.

Dr. Norwood [continuing]. Due to the large different variety number of payers.

Ms. Tolle. Right.

Dr. Norwood. We know that too. We think most of you have it and part of our thinking in this legislation we have here is that as you swipe a card through your computer and send it to Blue Cross and Blue Shield there is not any reason on a Class II or III that same information cannot go to Mr. McDonough.

Ms. Tolle. That is correct.

Dr. Norwood. There has to be--in our view, there has to be some single source in the State of Florida that is monitoring this if we are ever going to get a handle on it. And the question becomes, Mr. Chairman, who is entitled to know about that information? That scares us to death. I know it would be helpful to you, Doctor, to be able to monitor that particular data base and know and find out if your next patient got a Class II 2 days earlier. It would be helpful for you to know. On the other hand, if you did not then where is your liability. And who else gets to know in terms of HIPAA?

That is the problem that we are running into in trying to build this bill. If we can put privacy in it, and if we can limit the liability so that if for some reason the data you swipe through did not go through unintentionally then the next thing you know you are in court. I think we can solve this problem except that I do not know how to solve the Internet, and I am open to any suggestions. I think we can solve this problem if we can solve privacy and liability.

Ms. Tolle. Can I comment on Internet?

Dr. Norwood. If you have the answer, baby, I am ready.

Ms. Tolle. I do not necessarily have the answers but I have some friends from Florida Department of Law Enforcement here in the audience, and one of which I was speaking with last week when we had a drug symposium in Tallahassee, and again today. And he suggests to do reverse tracking on these sites. Where you can track the source where this medication is coming from. So you would need somebody who was well versed in tracking, much like a child pornography type of investigator, where you understand the computer science and you could follow those headers, and work backward. And maybe that would help solve some of the problem with these sites, I know that many of--I know it is multi-level, I understand that it is a really big process.

But that is one point that I have not heard brought up today, and I felt like it was definitely worthy of being mentioned.

Dr. Norwood. The problem is my 13 year old daughter goes on the Internet and types in a particular drug and sure enough, if she will just lie about her age, it is going to be filled and the way they do that is they have a rogue physician there that works at the site who signs every prescription.

We are trying to figure out how we can make them make sure that you sign the prescription without intruding too much, and causing you too much liability.

Ms. Tolle. We do have proposed language in Florida this year for Internet prescribing--for the Internet in particular

and that language requires a prescript--an actual physical assessment of the patient. A pharmacy is not allowed to fill a prescription based on an Internet questionnaire if they are aware that it is an Internet only questionnaire only.

Dr. Norwood. I know that you do have that, but that is going to bring down the wrath of God on us. You know, what we are trying to do is work with all parties here, and there is going to be a lot of parties that are not real happy that they have to answer to you about a physical before they prescribe. That may end up being the way it is dealt with, but it is certainly something that is going to cause a lot of grief trying to get 218 votes, I can tell you that.

Mr. Chairman, I thank you, and I yield back.

Mr. Souder. That is valid point, it is amazing what you can--if you would wait just a second, I have a question for you as well. I wanted to note that this is not that dissimilar in some ways from how we work with other narcotics. In other words, one way you look at where the production is, who is making the stuff that goes into the stuff, whether it is a controlled area or uncontrolled area. That can be problematic if it is not uncontrolled, but watching for leakage and slippage from the controlled area where it is being made, I understand Tanzania and other places like that, you look and see where the quantity, if it is not going to you, is there slippage there and are there other places that are being supplied.

And second would be the manufacturing of it, who is getting it and track those locations, and then, if indeed it winds up that because of restrictions here it goes outside like to India or other places, then the obvious delivery system becomes critical, because we are not going to be able to get it on the Internet, for the most part. We are going to have to get it in the delivery system, or the manufacturing or the growing.

The question I have for you is under current HIPAA and where does this go since we heard that many of these people are probably drug users, is that a criteria and is there a mandatory check to see if somebody has been picked up for a drug conviction before? And make that group if there are more prone to being addictive or seeking it for the wrong reason, why would that not be an automatic background check required in the prescription?

Dr. Norwood. Well, Mr. Chairman, I do not recall and I do not believe that is in HIPAA, but however----

Mr. Souder. Would it be prohibited?

Dr. Norwood. It is prohibited, among the other law already. Part of this is we have a lot of laws on the books, we do not enforce some of them. And the DEA--I am not as rough on them as John is, they will never have enough people to enforce this. There is no way on Earth that they could have enough people in the State of Florida to actually do what we need to do.

Mr. Souder. Dr. Berckes, when you as an anesthesiologist, do a background check, the person is asked whether or not they are using substances, the question is is there a background check to see if they have ever been arrested?

Dr. Berckes. No. In my practice and that is not the general practice, however, in cooperation with our Sheriff's Department and the detectives, we have had a real close working relationship. What I have is that every patient that walks into the office every time, not just the first time, they sign an affidavit in addition to me gathering the information that may



have changed since the last time they were in the office, whether that was the day before or a month before. They sign an affidavit that they have not received any other controlled substances from any other physicians or if they have, who that doctor is and what it is. That has worked really well because then when they sign that and we do all the legal stuff correct, then that is data that I guess the district attorney has been able to use for the prosecutor.

Dr. Norwood. Yeah they can, but you know--remember, this person who is in there to beat you out of this Percocet is going to burn your building down if you do not give it to him one way or the other. They are going to sign anything you say.

Dr. Berckes. They do. What I am saying is that has helped on the law enforcement end. But, there is no way that I can physically do a background check with any tool that is available now to know the veracity of the information that patient is given me. I mean there is a lot of things as far as the sniff test we can tell----

Mr. Souder. There are two types of things, that is why I thought we were maybe getting into HIPAA questions, because this is another type of way to address this, because some of these people may not be trying to beat the system, they may just have in the past used narcotics that shows in the risk assessment, that in fact they have a tendency to become more addicted, and not be able to get off. And they may not realize that even though--and they may not want to release that to you.

The question is and this is one of our pop up questions. Because we are having to get this for border control now, we are looking at when you get on an airplane, are there certain things that are basically in the system. It is a huge civil rights debate, but the question here is that you are also, protecting--we are not just looking for legal protection for the doctors, which we need to look for too. Because what people do not understand when you get sued it is not you who necessarily pays, it is everybody who comes to your practice who has to pay higher rates because of the malpractice insurance.

So, we have to do a lot of these things to protect you which is paperwork, and maybe--although most prosecutors probably do not waste their time on somebody who falsified a document, at least it is another level. The question is, that just seems like basic information, if risk assessment is that critical for the addiction and the danger, that you would have a pop up that would say that we can check and see who is an abuser. Now that is not necessarily an abuser of OxyContin. I was thinking more of the statistics that 2.8 times likely heroin, 1.7 cocaine, three times before have used, if we are picking it up in the autopsies, and if we are picking it up in the research, it seems like it ought to be something that ought to be much more restrictive at the beginning.

Because OxyContin, the difference--what I would put here is, yes all these other drugs may be at risk and it may shift. But this is not a maybe, what we heard from the DEA is they have never had anything that caused this much death.

Even though it also may be relieving more people of pain, if we can figure out how to manage those two questions and if there is a level of use; once it reaches an epidemic proportion and there is X number of deaths in society, all of a sudden civil liberties waiver on if you have been a narcotic. I was just wondering what we are running into, because I am not a

doctor, and I----

Dr. Norwood. Well, I do not think HIPAA envisioned that there would be a source of information on people's medical records that stores up the usage of narcotics. Having said that, I have no doubt in my mind that if we did do that, that somebody is going to read into HIPAA why it is against that.

Dr. Berckes. I just wanted to say I do this everyday, and I have been fooled. There is no way that anybody that does this can say you cannot be fooled that you cannot be scammed. But I want to dispel what I believe is the myth that writing one prescription of OxyContin or any other controlled substance, even if somebody who is genetically predisposed to drug abuse or addiction, that you are going to turn them into an addict. That is where the close monitoring of the drug and using the smallest hammer that you need and then ratcheting up as required. That is the only way that you are going to do it.

So, you can be fooled, but it is those tools and there is no substitute for that face-to-face looking at the patients seeing what they are doing and having them account for every pill. Can they scam you? Sure, but it cuts down on it drastically if they know they are being accounted for. And I can tell--it is hard to measure, but I can tell the people that come in that is all that they want. OK, and then they usually leave, yeah and the people working in my office, they are scared with some of these folks. And I am looking for their protection, but that does not keep us from the mission of what we are trying to do. And luckily, at least in my situation there is a close tie in with law enforcement.

What I have seen too much of I believe in the press is that you can have good intentions, write one prescription and you have turned somebody into a street drug addict. Sir, that does not occur. It is a continual misuse of medications. OK, the unbridled prescription without keeping track of what is going on, that is what leads to the problem.

Mr. Souder. Because many pain killers are prescribed for multiple use over a period of time, if you have a predisposition, you are more at risk than if you do not have a predisposition.

Dr. Berckes. Yes.

Mr. Souder. What I was kind of addressing is that it seems to me that you would get stopped for driving 62 miles an hour in a 55 zone. They can figure out what happens to you, why can we not when we are prescribing a potentially high risk addictive drug that can cost you your life, why can we not get this information that State cop has on the highway, about your past drug and alcohol addiction. It just seems like a disconnect.

Dr. Berckes. Right. And there is never too much information, and asking those questions is something that the prudent practitioner does. I mean we are required to, to practice good medicine.

Mr. Souder. You are asking the questions, but you do not have a way to verify it.

Dr. Berckes. But there is no way to check on the veracity of the answer, I mean, the whole doctor-patient relationship is predicated on trust and valid information. And how we can--there is no 100 percent way, there are subtle things you look at with a patient--the way they come in, what they are saying, who they are with, how they got to your office, these are all the subtle things that you have to look at, but we still are

going to be fooled.

But I am just concerned we already have a DEA, every doctor that prescribes narcotics in this country has a DEA number. So, it seems like we already have that data base, at least on the prescriber end.

So, I am interested in how are we going to--there is one way, there are two ways of monitoring it. It seems like we have the data base with the DEA, with the DEA number, Dr. Norwood. The DEA number you have on all the doctors in this country, we are all required to have DEA number.

So, that data base is there. But what is the information that we should be requiring and linking up in a national system for the patient. And that is where the HIPAA thing comes in. Because I tell you what, when I go online, OK, with my Bank of America account, here in Florida they know exactly what is going on in California, immediately. OK, and because it is that cross, I think the technology is there but I am concerned about folks that come into Florida. I mean it does not take long to get from the State of Washington to Florida. OK, and you think you are doing the right thing with the drugs and I would like to know, because if they are trying to scam me, they are not going to tell me well, what at 4 p.m., the pain doctor in Seattle gave them. OK, and then they are showing up in my office. I would love to have that information.

It just seems to be the privacy thing, but what are we going to use driver license number, Social Security numbers, you know we already have the prescribers with DEA. And what is the other thing, because whatever that other number is then we have fraud that is potential on that end. And that is where my biggest question is, and I think if we could address that, it is not a very sophisticated computer system that would need to figure it out. But it is who is going to look at it. I am asking the questions, I do not know, but it seems like we have it right here. And with respect to when we have a crisis, what do you do with a practitioner.

Well, I am chief of staff in a hospital, and when I have evidence that a physician is really out of line I am obligated and I have the legal ability to summarily suspend practice of that physician in that institution, until I get together all of the entities I need to see what is really going on. And we have a hearing process, and all the rest of it. And it seems to me that the Board of Medicine has a similar thing, but there seems to be a disconnect between the what is happening out in the street and the Board of Medicine. And then issuing, and how they can issue that appeal, that is not a Federal thing, but it seems like there could be Federal guidelines.

Mr. Souder. I thank you.

Dr. Henningfield. May I just add one part of the balloon that has not been directly touched on? And that is that one of the highest risk groups is young adults. And if we take a really long range view of this problem, we have to be looking at community efforts, we have to be looking at educational efforts. We know from our surveys that kids who have an increased perception of harm, that is a technical term, are less likely to abuse drugs. No kid should go to a party and have something offered and then be reassured that this is not a street drug, it is a prescription drug. Or what if they are reassured that it is not OxyContin, do not worry, it is something else? Kids should be getting a clear message from every source that using any prescription drug without a

prescription is potentially lethal, and that prescription pain killers can be as lethal and as addictive as any other drugs.

I have looked at the textbooks, this message it is not there, our system has not caught up. I do not think it takes a law to stimulate this. But working with Federal agencies like NIDA, and substance abuse prevention office of SAMHSA, you can encourage them to work more aggressively to get out the messages. And package them if you will, because the message here is a little trickier than it is for cocaine. The message for cocaine is easy, ``do not use any, any time from any source.'' With a prescription drug it is a more complicated message. And there is work there that our Federal agencies that have good people could do with encouragement and probably some funding from you.

Mr. Souder. I thank you for your testimony, we will probably have some additional written questions, if you want to submit any additional testimony. This stuff is very difficult, I know when this committee was actually divided into human service separate from the drug policy. Chris Shays was head of Subcommittee on Human Services and I was his vice chair, and we went through a number of things on the second use of drugs, which is the un-talked about huge thing in America, which is where the real kind of profit of the pharmaceutical companies often come from word of mouth, and hey, this works for this over here. And boosts the sales, and it is something that in our society it is very difficult to tackle the messages of what is safe and when.

Furthermore, our research on the interactive properties of these different types of both over-the-counter, yet alone prescription drugs. And trying to do this is very difficult, but when we have an epidemic level like we have had on one, it is an opportunity both to educate and help the public understand how best to manage it.

Well, thank you for your time, thank you for coming today.

Third panel come forward. Now if each of the witnesses will stand and raise their right hands.

[Witnesses sworn.]

Mr. Souder. Let the record show that each of the witnesses responded in the affirmative.

We thank you for your patience and as we do with all of the panels by tradition of the committee, the administration witnesses rise on the first panel, and then as the panel evolves we get more and more into the individuals and the individual practitioners and it has been a very helpful structure how we generally do this. I thank you for coming, Mr. Pauzar you are first.

STATEMENTS OF FREDERICK W. PAUZAR, FATHER; DOUGLAS DAVIES, M.D., MEDICAL DIRECTOR, STEWART-MARCHMAN CENTER; PAUL L. DOERING, M.S., DISTINGUISHED SERVICE PROFESSOR OF PHARMACY, UNIVERSITY OF FLORIDA; KAREN O. KAPLAN, M.P.H., SC.D., PRESIDENT AND CEO, LAST ACTS PARTNERSHIP; AND CHAD D. KOLLAS, M.D., MEDICAL DIRECTOR, PALLIATIVE MEDICINE, M.S. ANDERSON CANCER CENTER ORLANDO

Mr. Pauzar. Thank you, Chairman Souder, Representative Mica, Congressman Norwood, for the opportunity to testify here today.

My name is Fred Pauzar and I am the father of Chris Pauzar, a brilliant 22 year old who died from OxyContin 76 days ago,

just 2 days before Thanksgiving. The tragedy of losing a child is not something one should ever be forced to imagine, I will simply submit to you that the pain from this loss is so great, it overshadows nearly everything else in my life.

But each life that can be saved through the enactment of proper legislation and regulatory standards and procedures will be a life whose potential for greatness, whose contributions to mankind, may still be achieved. Each premature and needless death, such as that of my own son, is a heart-shattering occurrence that also deprives society of all the brilliance, all of the achievements, all of the greatness that will now never come to pass.

OxyContin was originally prescribed to my son for a minor injury to his shoulder. His frequency of dosage increased over time until he was taking 200 milligrams or more per day. All along, he was reassured that the long-term use of this drug was not harming him, both by his physician and by Purdue Pharma literature that suggested the appropriateness of prescribing OxyContin for pain that would be ``expected to persist for an extended period of time.''

When my son ultimately realized that he was addicted to this drug, experiencing flu-like symptoms and physical and emotional distress when he stopped using it, he needed and he sought regular therapy and medical support to detoxify, and to learn to live without Oxy in his life. Unfortunately, after breaking the pattern of daily use he wrongly decided to take it one more time, actually saying one more time would not kill me, the very evening that he died.

Since my son's death, I have been stunned by facts related to the marketing, prescribing, use and abuse of the drug that killed him. And I have been astounded that a clear and insidious correlation exists between the market penetration this drug has achieved and the toll of death it has left behind.

OxyContin came into existence in 1995, when according to U.S. District Judge Sidney Stein, Purdue Pharma deceived the U.S. Government by engaging in ``inequitable conduct before the Patent and Trademark Office'' in order to patent OxyContin. Its sales literally skyrocketed since, thanks in part to very aggressive marketing and the promulgation of performance claims that have not held up to scrutiny.

In 1995 and 1996 Oxy was sold as a chronic pain medication for use with cancer patients--very appropriate. Then in 1997, Purdue Pharma began to push this drug into a new market, such as back pain and injury. At the same time the company was reaching down into the broader market of moderate pain treatment, it added a more potent dosage, beginning the manufacture of 80 milligram tablets to complement the smaller 10, 20, and 40 milligram pills they were already producing, and so, by 1998, fully two-thirds of all Oxy prescriptions issued were for non-cancer pain.

Cleverly, Purdue Pharma paid for hundreds of physicians to travel on junkets where they were educated about the benefits of OxyContin, a Schedule II drug without a ceiling on allowable dosage. Meaning it is very difficult to decide when you are over-prescribing. Those physicians were, in the manner of a pyramid, told they would be paid speaker's fees for talking to other doctors about the benefits of OxyContin.

By 1999, Purdue Pharma's objectives included a reach toward one-half billion dollars in sales of their star drug, with

their marketing efforts targeting more consumer groups including seniors with direct to consumer advertising. It has been said that there was no DTC advertising and that is incorrect, because you could have walked into a number of different doctors' offices and seen placards in full color showing a grandfather with a grandson fishing in a stream, talking about how long term relief is at hand.

Again, while the marketing efforts sought to aggressively broaden market penetration, the manufacturing side of the company delivered an even more potent tablet once again, a 160 milligram pill.

By 2001, Purdue Pharma had comfortably rocketed past the \$1 billion mark in sales from this single drug, with the company noting in passing that the challenges presented by mounting evidence of OxyContin abuse in Florida, Maine, Ohio and other States, ``will continue to be a threat to the continued success of OxyContin tablets.''

In 2002, OxyContin sales hit the \$1.2 billion level, representing more than 80 percent of Purdue Pharma's total revenue, due in part to the advantage handed Purdue Pharma by our own FDA. As Purdue Pharma's marketing group noted in the face of mounting evidence that deaths in Florida and other States from OXYContin were exceeding deaths from heroin, despite what we were told earlier by the DEA representative. I am quoting now, ``It is unlikely that an opioid approved by the FDA in the future will have as broad of an indication as OxyContin now enjoys.''

The company knew that only too well. And in this regard Purdue Pharma is certainly correct. With the unwitting actions of many fine physicians who relied on the marketing promises made by an aggressive Purdue Pharma sales force, with the calculated and illicit actions of a small percentage of doctors who abused the system, and with a system that statewide and federally has been slow to communicate and to recognize the danger of this drug and to respond in an appropriate fashion, the daily death toll continues to mount.

In Florida alone, we can argue whether it is one person a day or 10 a day that die from this drug, but we know that the loss is truly incalculable but nonetheless devastating and real.

May you have the wisdom and the courage to deal effectively with this threat to our children and our society overall by taking effective steps now to monitor and curb the improper marketing and use of Oxy. And may you never know the pain that I along with thousands of parents before me and hundreds if not thousands more since, now feel.

Thank you, and I will be happy for your questions.

Mr. Souder. Well, thank you for sharing with us the pain that you feel in your family, and your trying to address the problems.

Dr. Douglas Davies is medical director of the Stewart-Marchman Center, thank you for being with us.

[The prepared statement of Mr. Pauzar follows:]

[GRAPHIC] [TIFF OMITTED] 95555.129

[GRAPHIC] [TIFF OMITTED] 95555.130

Dr. Davies. Good morning. Thank you for opportunity to address the panel.

The perspective I bring is one of a physician and I do have some pain management that I do as part of my practice. I worked

as an anesthesiologist for many years. Currently, I am an addictionologist in the University of Florida Department of Psychiatry, Division of Addiction Medicine. I also bring to you the perspective of being a person in recovery from the disease of opiate addiction.

As we have heard already, substantial quantities of prescription drugs are being illegally diverted in Florida, which results in a tremendous amount of death, it fuels the disease of addiction. Statewide the numbers I have seen included a 120 percent increase in treatment center admissions over the past 2 years for prescription opiates at our center. There is a summary of data available from Dr. Ernest Cantley the head of Stewart-Marchman showing more like a 400 percent increase in our admissions for treatments for opiates.

Diversion consumes State resources through associated medical expenses trying to take care of these people, through Medicaid fraud that we heard abundantly, and through treatment expenses if people are fortunate enough to make it to treatment. Prescription diversion certainly involves many scenarios--prescription fraud, illegal resale of prescriptions, doctor shopping, pharmacy shopping, and loose prescribing by practitioners characterized by the five Ds. Those are doctors that are duped, well-meaning physicians that who are simply getting slickered by patients looking for the drugs.

There are, on the other hand, dishonest practitioners. I know in my own community, my patients everyday tell me that so and so is a prescription mill, and so and so is a pill doctor. Physicians who are dated, who simply do not have adequate knowledge of how to--what are appropriate uses for these drugs. Physicians who for various reasons are dysfunctional, and simply cannot say no to patients, and physicians who are disabled by their own substance abuse issues.

Prescription drugs have overshadowed street drugs in several categories. In 2002, benzodiazepines accounted for more overdose deaths than cocaine. And in 2002, oxycodone, hydrocodone and methadone and benzodiazepines individually were involved in more overdose deaths than heroin. The problem is getting worse and there are abundant laws to deal with the perpetrators of prescription diversion. However, I believe it remains needlessly complicated to identify who these people are in the State of Florida.

When I have a patient sitting in front of me and I am being asked to perform an assessment to see whether or not they have a problem with prescription drugs, I have to spend hours on the telephone trying to call numerous pharmacies, assuming the patient is using his real name at the pharmacy and that he is even going to local pharmacies. Even when a patient reveals names of practitioners to me that are known to be pill doctors, it remains a daunting task as we heard earlier this morning to gather data on these people, and to investigate them.

Many other States do currently, and we have heard several numbers this morning 15 to 18 States at least currently have prescription monitoring systems. And in 2002, a GAO report described their effectiveness in reducing the diversion, by reducing inappropriate prescribing by practitioners and by serving a deterrent for doctor shopping, and by reducing the resources that have to be expended on investigation.

The current prescription validation program up for consideration in this State, would establish an electronic data base containing prescriptions of patients over the age of 16.

For it to make any sense it certainly need to cover all controlled drugs not just drugs in the higher schedules, but all controlled substances. It would make this information available to physicians, to pharmacists, to medical quality assurance personnel, and to law enforcement. And then some very simple requirements for reducing prescription fraud. It would require simply the quantities be written out, it is much harder to alter a prescription where all of the number quantities are written out, rather than stated in their numeral form. Require picture ID to pick up prescriptions. There is a typo here saying I recommend you use of counterfeit prescription forms, actually I recommend the use of counterfeit-proof prescription forms, and that this whole system would be administered by the Department of Health.

There is already a great deal of funding in place for this program. Purdue Pharma is said to be providing the State with \$2 million for the development of software to get this set up and the Department of Justice has also established a line of funding for this program. Certainly with the national scope of what we are talking about today this does need to be a national program. I know in the State of Florida this has been up for consideration for several years and shot down for several years. I certainly hope this is the year that is going to pass.

Thank you very much.

Mr. Souder. Thank you. For the record, for my information, but also, for those who reads the record is Stewart-Marchman Center a specialist center or general hospital treatment.

Dr. Davies. We provide all the addiction services for Volusia and Flagler County.

[The prepared statement of Dr. Davies follows:]

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[GRAPHIC] [TIFF OMITTED] 95555.132

[GRAPHIC] [TIFF OMITTED] 95555.133

Mr. Souder. Thank you. Next witness is Professor Paul Doering, a distinguished service professor of pharmacy practice, College of Pharmacy, University of Florida, who informed me that if his son had been playing for the Colts, they would have been in the Super Bowl rather than the Patriots. Unfortunately he switched teams.

Mr. Doering. To the Stealers.

Mr. Doering. Good afternoon, gentlemen, my name is Paul Doering and I am distinguished service professor of pharmacy practice at the College of Pharmacy, University of Florida, in Gainesville, FL. And it is my honor to be here this afternoon.

You know I went to pharmacy school in the 1960's and 1970's and they say if you were a member in the 1960's and the 1970's you were not there. I remember them vividly, because that was a time in which I came to the stark realization that the very same drugs that help people ease pain and make the suffering of surgery a little bit easier are the same ones that just as easily can cause severe injury and death when used inappropriately. This reality really hit home when I volunteered my time to assist in a methadone maintenance program for heroin addicts, a program that was being run out of Shands Hospital in Gainesville.

You know, in a strange sort of way, we as pharmacists are in denial: we do not like to admit that the very same



pharmaceutical drugs that might be the answer for one person's problem is the problem for the next person.

Working with heroin addicts and focusing on the drugs they used, is suddenly realized, kind of like a light bulb going on, that as a pharmacist I do know something about drug abuse after all. Since that time, I have been spending a substantial part of my career trying to help people to understand the downside risks that accompany the use of all drugs, but especially the recreational use of prescription drugs. Now, after all morphine is morphine is morphine, whether it is used to get high or used to relieve the pain of surgery. Its dangers are the same as are its bad effects when combined with alcohol or other drugs, and the risks associated with taking more medicine than prescribed.

Today, there has been a shift away from the abuse of so-called street drugs, more toward the pharmaceutical drugs. And although abuse of the OTC drugs is a growing problem, perhaps a point for discussion on another day, the problem of prescription drug diversion is what is wreaking havoc all across our nation. I will not repeat the statistics that you have heard over and over again, but we all agree that this is a huge problem.

It is especially a problem for pharmacists, because we find ourselves smack dab in the middle of this issue, and let me tell you why. The Code of Ethics of the American Pharmacists Association states, among other things the following: A pharmacist promotes the good of every patient in a confidential and compassionate, and confidential way. Pharmacists place concerns for the well-being at the center of professional practice. In doing so, a pharmacist considers needs stated by the patient as well as those defined by health science. A pharmacist is dedicated to protecting the dignity of the patient. And with a caring attitude and compassionate spirit a pharmacist focuses on serving the patient in a private and confidential manner.

Now, unfortunately, we spend an inordinate amount of time trying to sort out the patient presenting a narcotic script for some legitimate purpose from the patient who has obtained the prescription under false pretenses or who alters the prescription or outright forges the prescription for the purposes of abuse or resale. Unfortunately, most of us as pharmacists are not experts at handwriting analysis nor have we gone to the police academy to hone our skills at conducting an investigation. We are taught to trust the patients we serve and to be ``caring and compassionate'' as our Code of Ethics requires. Imagine our shock and frustration when a vial of pills from our pharmacy is found at the scene of a death investigation where a young adult has died from pills up and injected. Ours is a careful balancing act: while we want to keep drugs out of the hands of those who have no business having them, we must provide them with the caring attitude and compassionate spirit patients so rightly deserve.

One of the most valuable tools that we, as pharmacists have to combat the problem of drug diversion is open and honest communication. This includes communication between the patient, the doctor, the law enforcement community, and the regulatory boards of other health professionals. But unfortunately, while we do have laws in place to guide the pharmacist, sometimes laws can be difficult to apply on a daily basis. For example, Federal law tells us that the tenets of a lawful prescribing dictate that, to be lawful, a prescription for a controlled

substance must be: No. 1. Issued for a legitimate medical purpose. No 2. By an individual prescriber acting in the usual course of his professional practice. No. 3. And documented in the medical records.

Now, all this may sound straight forward but, we as pharmacists, have difficulty determining if the medication is ordered for a legitimate medical purpose. Furthermore, we may not know what constitutes the usual course of practice for one physician versus another type of specialist. And we almost never have access to the patient's medical record.

Looking at the problem from the patient's perspective, the therapeutic imperative should likely prevail. This theory compels the pharmacist to always dispense opioid analgesics when they are appropriate for a patient. On the other hand, the regulatory imperative commands us to never dispense opioid analgesics when they are inappropriate. And now matter how hard we try, no pharmacist can be faithful to both imperatives.

I think it would be wonderful if we had some technology that would allow us, for example, that somebody would give their fingerprint on some type of technology or pad that would validate and verify through some monitoring system. And I urge the adoption of such kind of system but only when the safeguards of confidentiality and privacy are indicated.

And I have longer comments that will appear in the record, and I appreciate your attention, today.

Mr. Souder. Thank you for coming and we will make sure the full statement is submitted and also, any additional materials.

Our next witness is Karen Kaplan, president and chief executive, Last Acts Partnership.

[The prepared statement of Mr. Doering follows:]

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[GRAPHIC] [TIFF OMITTED] 95555.137

Ms. Kaplan. Thank you, Mr. Chairman and members of the subcommittee, I am, as you said, Karen Kaplan, president and chief executive officer of Last Acts Partnership. Last Acts Partnership is a national not-for-profit organization that is dedicated to improving the care and caring near end of life.

You have heard compelling testimony today, and my message is one of balance. I appreciate the opportunity to testify concerning prescription medications, the opioid analgesics. You have heard they are controlled substances and they are controlled for good reason, but they are also indispensable medications for the relief of severe pain, especially pain near the end of life.

My remarks focus on the critically important need for balance, balance in the effort to address use, abuse, and diversion of the drugs. We must ensure that prescription pain medications are available to patients who need them even as we do all that we can to prevent these drugs from becoming a source of harm or abuse.

Under-treatment of pain is a major public health crisis. Medical experts agree that 90 to 95 percent of all serious pain can be safely and effectively treated. Yet, there is overwhelming evidence that under-treatment of pain is pervasive

throughout our health care system. Inadequately managed pain was reported by approximately 50 percent of seriously ill and dying hospitalized patients. In nursing homes nearly 300,000 patients are in pain on any given day as we are talking here today. More than 40 percent reported being in continuous pain for many months. The people who rely on these medications are our mothers and our fathers and they will be us.

We have made some progress in recognizing pain as a serious medical problem. For example, the Joint Commission on Accreditation of Healthcare Organizations added pain as the fifth vital sign, and you have heard about that already.

In 2000, Congress and the President declared this as the decade of pain control and research. So we must ask, with all the advances in pain medications and treatment, why is under-treatment of pain still so prevalent in the United States?

The answer is complex, but two major obstacles are particularly relevant to today's hearing. The first is a lack of physician education, a lack of physician education in palliative care. American medical schools provide little or no required education in palliative care according to a 2001 Institute of Medicine study. Only 1 of 125 medical schools are accredited by the AMA offered pain management as a separate course. This appalling situation must change if all physicians are to gain competency in pain management--and all must.

The second major obstacle to appropriate pain treatment is good physicians' fear of investigations by medical boards and law enforcement agencies, for prescribing opioids. This chilling effect was demonstrated by a recent survey of 1,400 New York State physicians, 30 to 40 percent of whom report that fear of regulators has influenced their prescribing practices.

Another face of this, a study of New York City pharmacies found that many, especially those in non-white neighborhoods, had inadequate supplies of commonly prescribed opioids. The reason cited by 20 percent of the understocked pharmacies in minority communities, was fear of investigations by the DEA. These practices based in fear can be found in every city, they may reduce some drug diversion, and abuse but they also condemn thousands of patients with intolerable pain to needless suffering.

Opioids are absolutely essential to good pain management, physicians must be knowledgeable about their use and should not hesitate to prescribe them when appropriate, for fear of reprimand or reprisal.

So, I return Mr. Chairman to the need for a balanced approach, one that recognizes the need to reduce abuse and diversions of these drugs but one that also recognizes that people in severe pain, particularly men, women and children with terminal conditions, must have access to medications that can ease their pain and help give them and their families peace.

In furtherance of this goal, Last Acts Partnership and 20 other national pain and health organizations joined the DEA in October 2001 to develop a consensus statement regarding prescription pain medications. It reads in part: ``Both health care professionals and law enforcement and regulatory personnel share a responsibility for ensuring prescription pain medications are available to the patients who need them and for preventing these drugs from becoming a source of harm or abuse. We all must ensure that accurate information about both the legitimate use and the abuse of prescription pain medication is

made available. The roles of both health professional and the law enforcement personnel in maintaining this balance is critical.'

This statement is attached to my testimony, it has been disseminated widely, used in many different settings. There are now 42 organizations participating in what is known as the Pain Forum. Many also belong to the RX Alliance chaired by former Mayor Guiliani, also looking for ways to invigorate balanced approaches.

We continue to seek ways to advance this dialog, and to provide a comprehensive answer to this. We have recently developed and will be publishing shortly a question and answer guide for non-pain specialists, physicians, pharmacists, and law enforcement personnel.

I applaud your work here today, I appreciate the opportunity to testify, and would be happy to answer any questions you have.

Mr. Souder. Thank you.

Our clean-up hitter for today, is Dr. Kollas, who is medical director, Palliative Medicine--in Indiana, anything over five words we have to wrestle with--Head of the M.D. Anderson Cancer Center in Orlando, in Orlando Regional Health Care.

[The prepared statement of Ms. Kaplan follows:]

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Dr. Kollas. Thank you. On behalf of the Cancer Center and Orlando Regional Healthcare I want to thank Chairman Souder, and the subcommittee for inviting me to testify today. I would also like to thank Representative Mica and his office for their support and thank those who contributed to the research that I will be presenting in part.

My testimony will focus on the views of cancer patients regarding their experiences with pain medications. My goal is to give them a voice in this subcommittee's discussions. We surveyed 1,200 randomly identified patients who received care at the M.D. Anderson Cancer Center Orlando, between August and November 2003. The details of the methodology are available in the written testimony that I submitted earlier.

I want to point out that 52 percent of cancer patients reported that they experienced pain daily; 41 percent agreed that pain interfered with their ability to work and be productive; 20 percent felt that they could not preform routine activities, these include getting dressed, driving the car, shopping for groceries due to pain; 43 percent of them expressed concerns about using pain medication because its potential for addiction. I would also note that of those patients who had concerns about addiction, they reported pain twice as often as those without concerns.

The results confirmed that many cancer patients suffer from pain on a daily basis, and that it affects the ability to live their lives in a free and productive manner. With regard to

OxyContin and their pain experience, about 41 percent of the respondents had used OxyContin to manage their pain, whereas 59 percent reported using other opiate analgesics for their pain. In the first group, 82 percent reported the OxyContin relieved their pain, but 72 percent in the latter group responded that they received pain relief with other opiate medications. Additionally, 53 percent of those taking any opiates agreed that opiate analgesics were the only medications that helped their pain.

These results suggest that opiate analgesics offer effective relief for cancer pain even when other analgesics failed. They also suggest that some cancer patients may have better control with OxyContin than with other opiates, although I would strongly caution the committee that this was not intended as a formal comparison of pain medications. And rather reflects the view of the patients that we surveyed.

Additionally we asked some questions about the cancer patients' experience with the media and OxyContin, 43 percent disagreed that the media had adequately addressed the issue of cancer pain, but we found no relationship between concerns about addiction and attention to media coverage. Given this, I would suspect that cancer patients value their own pain experience more than what they read, hear, or view in the media. Fear of OxyContin or other opiate analgesics is a complex multi-factorial phenomenon, not simply the result of intense media coverage.

This subcommittee has accepted the challenge of preventing diversion and abuse of prescription medication while preserving legitimate access to those medications. Our survey of cancer patients reaffirms that opiate analgesics, including OxyContin, offer relief for pain often more effectively than non-opiate analgesics. In spite of media attention to prescription pain medicines, cancer patients seem to base their opinions of opiate analgesics on their own experiences.

In light of our patients' view, I would offer several guiding recommendations to the subcommittee regarding its mission. Because cancer patients need pain medication, we would discourage regulatory efforts that would reduce legitimate access to opiate analgesics, including sustained release oxycodone. However, we recognize clearly that the government has an obligation to protect those who suffer from the diversion of use of analgesics.

I would applaud this subcommittee's efforts to develop regulatory mechanisms that would protect these people. I would also remind the subcommittee that those who misuse prescription medications often suffer from underlying untreated psychiatric illnesses that influence their drug abuse. Successful solutions to the problem of diversion and abuse should take this phenomenon into account.

Last, I would encourage the subcommittee to continue challenging medical professionals to help create new policy through frank discussions. We believe that education in pain management helps medical providers to recognize and avoid diversion or misuse of prescription drugs. I would add at this point that I feel medical providers should welcome the opportunity and the responsibility to serve in this battle to help prevent misuse and diversion of prescription drugs.

I would strongly encourage the development of other strategies that emphasize an educational approach, and I would specifically cite House Resolution 1863, the National Pain Care

Policy Act of 2003.

I would also note that electronic monitoring which is being considered in Florida has shown to be effective in other States, including a specific example of Connecticut. The only concern I have with regard to electronic monitoring has to do with HIPAA violations, and we have talked about some of those issues, at least in a preliminary fashion, today.

Although the subcommittee faces formidable challenges, I conclude my testimony on a positive note. When we mailed our surveys, we hoped that our patients would entrust their voice to us, and they did so. They embraced the belief that their views and concerns would reach your ears, and now they have. Although we face a difficult task, we face it openly and with resolve to succeed. Because of this, I have renewed hope for a better future for all patients in pain, and I would be very happy to entertain your questions.

Thank you.

[The prepared statement of Dr. Kollas follows:]

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Mr. Souder. Well let me start off with just a couple of things to clarify for me, since I am medically challenged. My wife is an occupational therapist and she does the thinking in this area, and I kind of wander in and she is always kind of envious that I am at the hearings and she thinks that I am a ignoramus on the subject and she knows the details. But you gradually pick up bits and pieces, just enough to be dangerous. But I want to clarify a couple of things.

My mother-in-law recently died of cancer. Her pain definitely was greater in the last stages than it was earlier, is that usually true?

Dr. Kollas. It can be, we see that commonly. It depends on the cancer.

Mr. Souder. But it is not always true?

Dr. Kollas. It is not always true, but it is true very often.

Mr. Souder. And so, would the pain killer use likely escalate as you go through cancer treatment, or increasingly is the same thing being prescribed all the way through?

Dr. Kollas. No, the use of the medication may escalate. Actually you bring up a point that I wanted to make earlier. Physicians are sort of used to dealing in population medicine, it is what they teach us in medical school. They want us to view people in categories of diseases if you will. So we think

of people as having hypertension, or we think of them having diabetes, or we think of them having cancer.

To do good pain management you have to abandon that view somewhat and look at people as individuals. Every one is different. So the right dose of a pain medication for one person may not be the right dose for another patient.

Certainly, you are going to see general trends, and it is not uncommon for patients with cancer at the end to have more difficulty with pain. And in fact in my experience, the few people that we have seen on dosages of pain medication of opiate medication that would stagger the subcommittee's members all occurred related to end of life care. Given that, that is why it is hard to answer that question, it depends upon the individual patient. And it also raises the importance how physicians need to be trained to take that into account. It is a very different approach than what we learned in medical school, where it is very disease based. We try to look at-- palliative medicine particularly is focused on relieving suffering in multiple dimensions, and that is a very different approach.

Legislation that would encourage that type of education is extremely important and I would argue that physicians should be asking to be empowered to take a more active role in this, to help prevent misuse and diversion medications, because clearly the more you know, the better you are able to do those things. We might get fooled by patients once in a while, but it is a lot tougher to be fooled by a patient when you know more about what the techniques are used to divert medications.

Mr. Souder. If a cancer patient is younger and mobile even if it may be likely failed, is the mere factor of their mobility, their ability to hold a job--well, let me first ask a fundamental question about OxyContin.

Dr. Kollas. Sure.

Mr. Souder. Does this impact your ability to do certain types of work if you are taking a dose?

Dr. Kollas. It can, it is individualized. Let me give you an example, I have a patient who is 48 years old. She has metastatic breast cancer. I asked today--I did not ask today but I asked if I could discuss her case with you today. I saw her in the hospital about 2 weeks ago, she was having a stabilization surgery to help her spine, because she has metastatic disease to her spine. At any rate, she works for one of the technical companies that is based in the Orlando area. She has been able to continue working at her job, awake and alert despite the fact that she takes 640 milligrams of oxycodone every 6 hours. When she gets a refill prescription and she goes to the pharmacist, she tells me I am very scared because look at all the tablets that I take. Yet, she is awake and alert.

Now, when she comes to see me, I document that in my note, I do a physical examination. The physical burden of her cancer is just tremendous, I mean the surgery that she underwent is a laminectomy, she had a spinal fusion involving four segments of her spine. Afterwards we actually had to convert her from oral medication to medication that she could use intravenously, using a portable pump. Because she is to the point where literally it becomes a physical problem to have to take that many pills. They could get stuck together and cause her to have a intestinal obstruction.

So, when you ask me the question are people able to

function cognitively when they take OxyContin, my answer is yes, but everybody is different. Some patients do better than others.

Mr. Souder. Let me ask, are there restrictions in driving in Florida?

Dr. Kollas. Yes, there are restrictions in driving in Florida.

Mr. Souder. Is it not also true that alcohol has a different impact on different people?

Dr. Kollas. Absolutely.

Mr. Souder. And yet, our laws that regulate do not respect that difference. In other words, we do not say some people can handle three beers, and some people can handle two beers because they have to protect on the whole.

Dr. Kollas. Sure.

Mr. Souder. Would you not agree, and one of the things--to me, this debate is not predominately about people at the end of life or who are probably--in other words, when we dealt with certain waivers, for side effects on AIDS, for AIDS patients--

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Dr. Kollas. Right.

Mr. Souder [continuing]. We basically said they are dying, if they are willing to take the side effects, because they are dying.

Dr. Kollas. Sure.

Mr. Souder. The question here is that predominately on the moderate pain, or other types of things other than cancer. While it is a concern that we do not pass laws--but quite frankly, one thing, Ms. Kaplan, that you can probably be relieved of after today is that doctors and pharmacists do not have to worry about being prosecuted by DEA, that if anything for them to use that as excuse, simply is not valid around the country.

One of the things I wondered, if I can take it along--I wanted to put that point into the record that I do not view this hearing as predominately related to the cancer, or the highest risk, or where the pain is greatest. I view this as we are trying to identify in the middle and I would like to have one more comment. I also, wonder, Ms. Kaplan, whether there is a concern of the people who say that they are worried about prescribing, whether you have discovered they are worried about being sued. I would assume there is more concerns about the losses and the malpractice then there is about the DEA, because, the fact is that we are not doing that much in the country on law enforcement.

Ms. Kaplan. I think that I would agree that the issue of the chilling effect may be largely a perception issue, and requires some fairly active public education on the part of the DEA, and they are indeed addressing that issue.

In terms of the second part if you would restate the second part of----

Mr. Souder. Do you not believe that one of the things that causes doctors not to prescribe is that they are concerned about lawsuits?

Ms. Kaplan. I think that is not the case in this situation, doctors in fact are being sued successfully for under-treatment of pain. So that should be a push in the other direction. There clearly is a malpractice crisis in the country. I do not think this plays--fear of over-treating plays a large role anymore in that.



Mr. Souder. That is kind of a different angle on it. Mr. Mica.

Mr. Mica. Well, first I want to thank Fred Pauzar. I have known Fred for a number of years through business, I cannot imagine the pain and the absolute incredible loss he has experienced and there are other parents and loved ones out here that have lost people they care about.

This hearing is not going to bring anyone back. What it will do and I compliment you Fred and others who pursue this, is to try to get government to respond to a situation of prescription drug abuse, and bad people who have also gamed the system and caused untold pain, and created an incredible challenge for us. Unfortunately, I have known too many parents, I know Fred, and I have known others who have lost their children in the community. I could name names of parents of kids, I hope I do not have to do another one of these hearings ever, or request a hearing like this.

But it is sort of a challenge of our times, this is what-- we are talking about this particular narcotic that is available since 1995. We were talking about that earlier, how long has it been available, and then if you look at the statistics, they are off the chart. I read--I knew the problem, and I read the same day of Chris's death that we announced the hearing. Again, nothing is going to bring back your son or some of the others, but from this hearing and from your very admirable efforts, hopefully we can bring some of this situation under control.

And this is the process that works, sorting it out, work with my colleague, Dr. Norwood, to have legislation pending, and I have learned that there are other proposals before Congress, and maybe we can craft something. It is also obvious that people do need remedies for pain. I have been through the same thing, Mr. Souder, with family members that have passed away in the last couple of years, and had to endure incredible pain and seeking relief. We want to achieve a balance, but we also want to achieve a protection so that we do not have anyone suffer the way some of the folks who came out today have.

So, again, not so much as a question, but a statement to say thank you for your testimony.

From the pharmaceutical standpoint, again, I think we are trying to achieve a balance and protection and some system. I do not know if you were here, when I relayed that we had several demonstrations projects in the Medicaid area to try to come up with software that will resolve this. Are you familiar with any of those.

Dr. Doering. Yeah, as a matter of fact one of the things I did not tell you another hat that I wear, I do a lot of consulting work in cases that are being prosecuted. The one that Mr. McDonough talked about earlier in Pensacola, I testified twice in that case. I was involved in several cases close by and I remarked to one of my colleagues at the break that it was interesting that a current case that I am working on in the panhandle was brought forth by Medicaid fraud.

Now, you do not typically think of them as, or I do not, as the enforcement arm in criminal activities involving narcotic drugs. But it is the Medicaid fraud, and apparently they have a system that others do not, where they can look on paper and say whether it is, wow, look how much we are spending or wow, look how much they are prescribing. But that current case has evolved into a well-coordinated multi-jurisdictional type of task force.

Now, as you well know, prosecuting these kinds of cases is lengthy, it is costly, and sometimes people are falsely accused. I have a new respect for the legal system. I was a consultant in a case with DEA that just pled actually a doctor there in Arizona; Phoenix, AZ; Tuscon, AZ. And I do not want to tell these taxpayers how much of their money was spent that I know that on April 15 that is going to be a large part of expenditure. Is it worthwhile? Absolutely. If one bad doctor, one bad pharmacist it taken off the street, it is worth the effort.

But, you know, I believe in the 80/20 rule. I believe that these 12 prescribers that we heard about earlier today, I mean if they are really accounting for that much of the diversion and the bad prescribing and the deadly use of these drugs, that is where the focus ought to be. I learned a long time ago, you look where the light is, and if that is where the light is, I mean with all due respect to my colleagues on the left here who made a very convincing presentation, I do not think that is where the light is. I think the light is with people who are either fully educated who are cradled with the D's that you mentioned, that are criminally involved. We have to take them off the streets and put them in jail.

Dr. Kollas. May I just add something?

Mr. Mica. You want to respond?

Dr. Kollas. One of the points that I wanted to emphasize is just that. Realize that I am involved in treating a group of patients, when I say I relieve their pain it has the same sort of analogy that I would use for a politician kissing a baby. You make cancer patients' pain better, people are going to say that is a good thing. That is pretty close to a no-brainer. I think there is a problem with physician involvement in diversion and misuse of these medications. You guys keep talking about these 12 physicians in Medicaid. I live in Florida, so I get to read the paper and one the physicians that they were talking about was writing prescriptions for patients who were dead.

Please hold the physicians accountable when they do this. That is clearly criminal, and it gives everyone else who is trying to do an honorable job of this, a bad name. And it is difficult enough, I mean, you know, looking at people in an individualized fashion is very labor intensive, it is important. I am very passionate about what I do and I view it as an honor and privilege to be able to do it. But, please when you see physicians that are clearly doing it related to obtaining money or obtaining some other favor for writing a prescription, put them in jail. We will be safer and we would not have to have these meetings anymore.

Mr. Souder. Dr. Norwood.

Dr. Norwood. Mr. Chairman, you are to be commended on this hearing, and especially for the witnesses that we have had testify this morning. I think it has become very clear to all of us in the room and all of us on the dias up here that this is a very complex, it is a very difficult problem.

All of us are in great sympathy with you, Mr. Fred Pauzar, and want to do anything we can to see that kind of thing cannot happen again.

On the same token, Ms. Kaplan, I associate with your remarks a lot, what you are saying about under-prescribing for pain is equally important, and it is particular important if it is your mother dying of cancer. It gets to be a lot bigger

subject matter at that point. I am in great sympathy with the majority of physicians who get their profession black-balled because of some 10, 12, whatever the number is really, really, bad people in my view. I agree with you, Doctor, they would serve out the rest of their days practicing medicine in prison. Those that would violate the Hippocratic Oath I do not think very much of, is probably the best way I can say it without the chairman having a fit.

But the poor physician is caught in the process of if I do I get sued; if I do not, I get sued, and that is not a good situation.

I associate with your remarks when you are talking about the code that pharmacists have to live by in dealing with confidentiality. That is going to be one of the real difficult problems with us in dealing with this problem. Obviously, if we are going to solve it, somebody has to have a data bank. I do not think the Federal Government needs a data bank, but I think Florida does, and I think they need to be able to talk to the data bank in Georgia, because you can run back and forth between Tallahassee and Valdosta and load up.

But who actually gets to go into that data bank. The liability questions of that are gigantic, and very difficult to solve.

Last, Doc, what do you do, I know you know--you know who the pill shops are. I know in my town, or I used to when I was really into all this. What do you do with that information, when you know that?

Dr. Davies. I do not do a whole lot with it, right now.

Dr. Norwood. Why not?

Dr. Davies. It would just be--I do not know if there is a forum to go to with it. The State rules, the laws are not real clear to me. And the source of my data--there is so much stigma around addiction and around addicts, although plenty of my patients are us, they are not street level addicts.

Dr. Norwood. You do not have to do the investigation. Are you not morally responsible to at least let DEA know something is going on here that is wrong. It is their job to do the investigation. And it is the court's job to make the determination of innocence or guilt. But should you not call up your DEA folks, and say something is not right over here on Third Street.

Dr. Davies. I would feel a lot better about that if I had access to real data and real numbers. And not just what they are going to tell me is hearsay from patients. I mean, I have a great concern about it and that is precisely why I brought it up.

Dr. Norwood. I knew you did, and I am not trying to criticize you about this, I am just saying that you guys know, I know you know. You may not have proof but that is not your job. But you know what is going on out there in your community, you know who the bad guys are, and all I am saying is spread the word. Let those agencies that are responsible for dealing with that, deal with that. But there are so few employees at the DEA, if they do not get a little help from us out in the field, if we do not direct them a little bit, when we know bad guys are out there, it just takes them that much longer if they ever catch them and stop them. And if the people are not guilty, fine. That is what the whole system--that is what our justice system is all about.

Mr. Chairman, I just congratulate you. There are a couple

of bills going around, being worked on in Washington and they do not all necessarily take the same course, but all of them involve data collection, so somewhere out there we can find out who is prescribing what. Some people want to do it on a Federal level, I do not fit into that category. I really think it is more of a State thing.

But I pledge to work with you and Mr. Mica to do whatever we can do there to solve this problem.

Mr. Souder. I want to thank Congressman Mica for being persistent in raising the subject and making sure we had this hearing, to Mr. Norwood, for his leadership in the area, and both of them for their chairmanship in multiple areas in Congress.

I want to make sure that in the record we note a couple of other things.

First, Dr. Davies, I really like the five D's because it illustrates how this is not one solution. In other words, for the data, that is clearly an education effort in the form of HHS and other institutions doing more to get the information out. We have heard a lot about that today. But the duped, the dishonest, the dysfunctional, disabled all require different approaches. There may be some clustering and all those are part of this problem.

I think a hearing like this helped us clarify where some of the targets should be in larger targeting. We do not know that all 12 of those individuals are guilty of any violation, they may in fact have more Medicaid patients, which may be that is why they were among that. They may be among the inner city urban area, for example. There were certain suggestions implied that they certainly should be the places you start. That there are certain things you might look at at the Federal level, but in that as our committee having both authorizing and direct oversight over the national ad campaign, over ONDCP and HIDTAs, we understand that DEA and our dollars are stretched very narrowly and that the south border right now is so porous that much of that has to be focused on and the Caribbean. And we cannot go off into each new hot thing that is the focus, and divert large amounts or we will get none of them licked. We have to kind of focus in but we also have to have secondary efforts in emerging threat efforts inside that. And we are helping identify that with this hearing.

But let me say something and end this on a less than comfortable note. That fact is what Mr. Pauzar raised was more complicated, and that was not just about massive diversions, not just about people who were former addicts, who use this which make them higher risk, not just about big abusers. But are there risks to individuals, because we are going down to moderate use, which is much more explosive than what we can agree on here, and we have obligations in our society to look at some of the traditional ways of prescribing. The secondary use of those drugs, the interaction of those drugs, and the dependencies and risks that are occurring beyond the kind of OK, these 12 people are terrible, because your son probably was not getting it from 1 of those 12 people. He probably was not a previous addict, and then all of a sudden he is dead, and we have another class here that is much more complicated, he was not dying of cancer, and these, this zone is really where the political difficulty comes. We will probably be able to address the more egregious things. Do you want to add something?

Mr. Pauzar. Mr. Chairman, thank you for that. You are

correct, my son was not a drug addict, and he was not taking prescriptions that he obtained from 1 of these so-called 12. Twelve is an arbitrary line that was drawn, simply because the gross magnitude of the quantity of prescriptions that were being written presumably illicitly by those 12 doctors was so egregiously horrific that it stands out. But that does not mean the number is 12, the number may be 100, it may be 20. There are a number--a small number fortunately, a minority of physicians who are over-prescribing and prescribing inappropriately.

But your remarks that this is a very complex situation is very apt. The solution is not one thing. It is not going to be a tracking bill, that requires tracking. It is not going to be more dollars for DEA, or better education for people at DEA about what really is going on in some of the burgeoning new markets of drugs, illicit drugs and prescription drugs that are being abused. It is a very complex three dimensional puzzle and it requires communication between the agencies, and it requires action to be taken legislatively, and it requires action to be taken on a State level too, where the boards of medicine and others are regulating the physicians.

Because it will not stop; simply to track the information and to know that it is there, is not an answer. We had an awful lot of data before we lost our last space shuttle, but that data just was not analyzed and it was not acted on correctly. So, the organizations that are vested right now with power and with a mandate to act, have to be informed and they have to communicate with one another, and there has to be stronger teeth in the legislative attempts that you take. And certainly drugs like OxyContin have to be taken away from moderate pain relief, because if anything has been shown here today, that has been talked about today by everyone is that we do not want to deprive terminal cancer patients of OxyContin. You do not want to deprive people who are severely afflicted with pain from those arsenals that are available to them to deal with that pain to make their lives manageable, but you want to take people who can take Tylenol instead and make sure they never receive a script, that they are never given a prescription for something that might well kill them as it did my son.

So, it is an extraordinary complex problem, and I appreciate your attention on this but I also appreciate the fact that it is going to take a lot more than this hearing and a lot more than one piece of legislation to cure it. But every day that goes by just in this State alone, I am not sure--is it one person who dies Congressman Mica or is it 10, in Florida? I know that what we have, based upon the statistics that we see, even in the time we have been talking here, there had probably been one to two deaths in the State, in this State alone, from OxyContin or oxycodone. So, I am enormously distressed by the problem because of my own loss, but I am more distressed, and believe it or not I am more distressed by what I see tomorrow. Because every day that goes by without decisive action means that there are more parents like myself.

Thank you.

Dr. Kollas. I just wanted to add something to that, and I hate to add another layer of complexity on your task. Using OxyContin, for example, for moderate pain, on the surface it seems to be something that is a bad idea, we should not do that there are other medications available. What I would do is caution you when you approach it that way. There are over-the-

counter medicines that are every bit as lethal as OxyContin, people have not chosen to abuse them because they may not have the same sort of effects that opiate medications do. But if you take more than 8 grams of Tylenol you can die from liver disease. If you take too much Advil you can die from renal failure. Sometimes you are forced to use medication for moderate pain when you would rather use something else. If somebody has difficulty with renal insufficiency than a morphine-based medicine might not be the best choice for them when they have moderate pain, they may have an allergy. If they are hemophiliac they may not be able to take medicines that aspirin or that are nonsteroidal anti-inflammatory drugs.

The point that I want drive home to the panelists is that there is a certain level of expertise that is involved in pain management. You know I went to medical school, I know what a cardiac catheterization is, I know what they do when they do the procedure. I am not a cardiologist, I do not do them. You would be nuts to let me do a cardiac cath on you, OK. What I do know is that I have special training that allows me to handle something that is medically sophisticated, that many of my colleagues do not have. So, I really think that part of what you need to consider is, who is able to prescribe these medicines, and what is their amount of training and if it is that all doctors should be prescribing pain medicine because pain is such a broad problem, then all doctors need more education in pain medication and in pain management. And if you want to say there is specialized cases in pain management that requires special expertise then it would be wise to recognize that. It would be wonderful if there was American Board of Medical Specialties acknowledgment of palliative medicine as a specialty. There is not yet. I would love to see that happen and I think that would go a long way to help with some of these issues.

But understand that this is an important area of medicine that is more complex than--I think you have an appreciation of it, but it is more complex than you even realize. And please use the resources--clearly from today's hearing, there are many resources available to you and we are all committed to making this problem better.

Mr. Souder. Well, I appreciate those comments and as somebody who is, as I have said several times, a very strong supporter of doctors and the medical industry as a whole, let me again make this statement. Everybody would like best to be left alone, small businesses would like to be left alone, everybody would like to be left alone. And I know that in health care this is something we hear on abortion law, it should be between patient and doctor, but you know what when society makes the decision, there are restrictions on it. Same with illegal narcotics, and there is a point, for example, as we try to work through incredibly difficult issues in Medicare, Medicaid payments and now the private sector mimics it. What kind of health insurance--Dr. Norwood, has been involved in this, trying to redo health care since he has gotten elected. And we run into lawsuit questions, where do we make compromises. But when the Federal Government crosses the point where we are carrying most of the health care cost than the private sector, which was not doing cross transfer and now all of a sudden you have HMOs and others who are necessarily already restricting the medical profession to make necessarily the kind of in depth consultative type traditional, this is my

doctor, this is the patient, where you are trying to run lots of different people through where there is not heavy background checks. And then all of a sudden, we have an explosion of 10 deaths of a day in the State of Florida, related to one thing, I am sorry, it is not just doctor-patient anymore. It is a lot more complicated than that, and we need to make sure we do not overreact and overstate it.

But there are going to be controls, because of who is paying for it, because of the reactions in society and then we have to make sure that we do not do irreparable harm to others who are benefiting, but we have heard testimony today that this has had greater than any other prescribed drug in the number of deaths. So to not act, suggests some irresponsibility. And one of those things is to look at yes, moderate pain is something that requires maybe certain waivers. We should not make it so blanket, but it is not something that--we are not living in a just leave us alone world at this point. And no group likes that, and I think we have as great a danger of over-regulation as under-regulation, but at a certain point you say this as reached the point, a threshold where action is going to be required. And I would say that clearly this coming.

Now one other thing, we are dealing with ephedrine and things that go into aspirin and so on are some of the main components of meth labs and clearly even as Mr. Doering said, look there are over-the-counter problems right now too, it is not just prescription. We are going to deal with it more, and quite frankly, the more successful we are at controlling our borders, the more problem we are going to have with domestic produced drug questions, and that is why we have to get into prevention programs, treatment programs, of all type. But at the same time that means that there is going to be more pressure with our addiction problems in the United States, unless we more effectively communicate the dangers of getting, as we have heard multiple times, warning people about the interaction, unless the drug companies get more aggressive and unless the pharmacies rather than just say trust, trust, but verify and unless the doctors do trust but verify. This is not Marcus Welby M.D., and I know the younger people do not even know what I was talking about.

Things have changed and we all need to change with it and helping make sure, hey look, we like over-simplifying government, we have to deal with laws that reach broadly, not an individual law for each case, so we have to balance that, but we are going to have to do that. I am tending to go on here.

Any additional statements you want to get in, you can submit them for the record, we will probably have some additional questions.

Once again, I thank Mr. Mica, and Mr. Norwood, thank everybody here for their patience as we went through this hearing.

With that, the subcommittee stands adjourned.

[Whereupon, at 1:30 p.m., the subcommittee was adjourned.]

[The prepared statement of Hon. Dave Weldon and additional information submitted for the hearing record follows:]

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